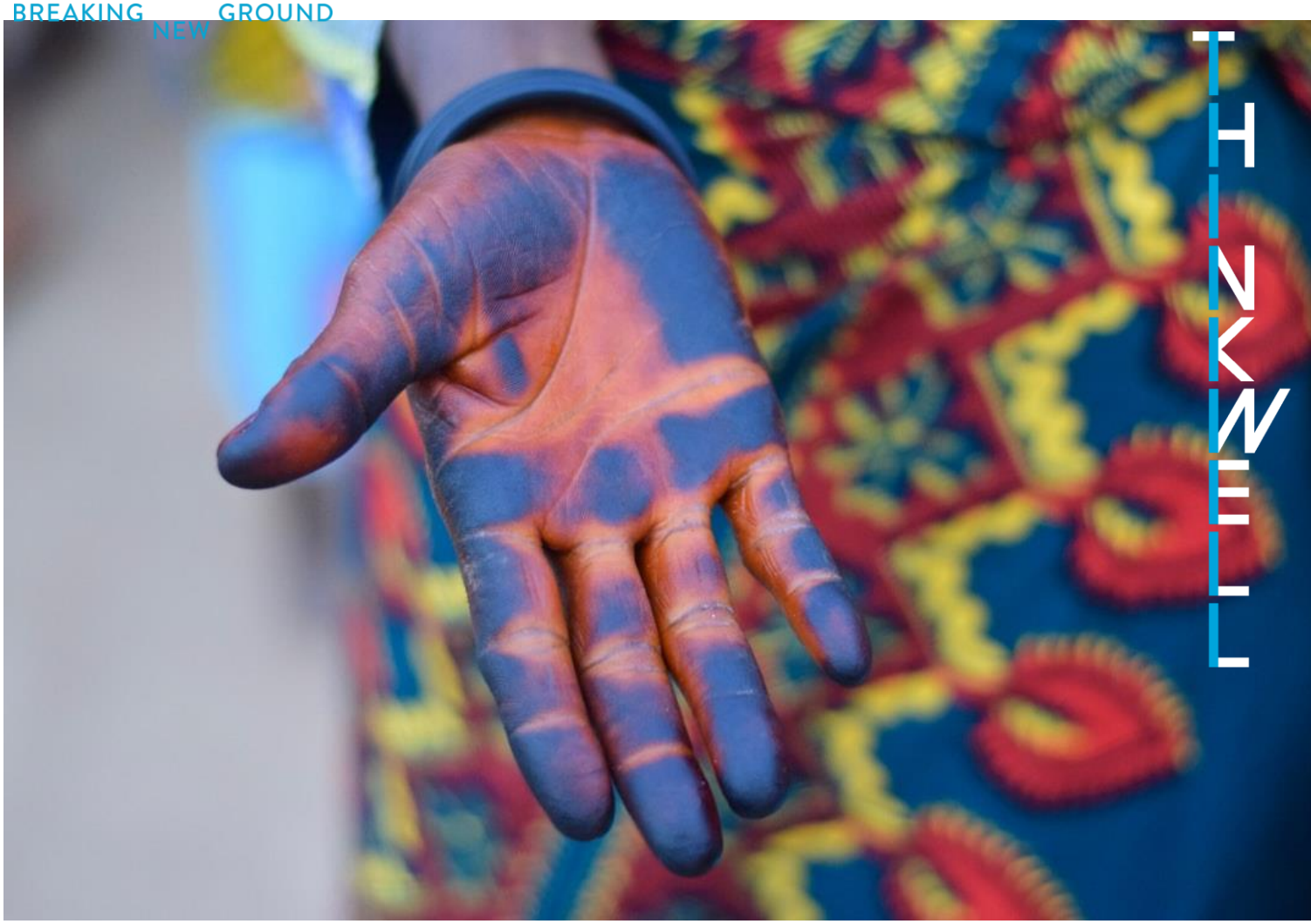
CASE STUDY

Intergovernmental Rivalry and Fragmentation: How Federalism Shapes Public Financial Management and Health Financing in Nigeria

April 2022

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For more information, please visit our website at **https://thinkwell.global/projects/sp4phc/**.

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ABBREVIATIONS

BHCPF	Basic Health Care Provision Fund
BMPHS	Basic Minimum Package of Health Services
CHE	current health expenditure
DFF	decentralized facility financing
EMT	emergency medical treatment
EXT	health expenditure from external sources
FCT	federal capital territory
FMOF	Federal Ministry of Finance
FMOH	Federal Ministry of Health
GDP	gross domestic product
GFF	Global Financing Facility for Every Woman Every Child
GGE	gross government expenditure
GGHE	gross government health expenditure
GIFMIS	General Integrated Financial Management Information System
GOE	government-owned enterprise
HMOs	health maintenance organizations
IGR	internally generated revenue
LGA	local government area
MDAs	ministries, departments, and agencies
MTEF	medium-term expenditure framework
MTSS	medium-term sector strategy
NGN	Naira
NHIS	National Health Insurance Scheme
NPHCDA	National Primary Health Care Development Agency
OOP	out-of-pocket
PBF	performance-based financing
PFM	public financial management
PHC	primary health care
SLAMs	State-led Accountability Mechanisms
SSHIS	state social health insurance scheme(s)
SMOH	State Ministry of Health
SPHCDA	State Primary Health Care Development Agency
THE	total health expenditure
UHC	universal health coverage
VAT	value-added tax

INTRODUCTION

Over the past five decades, sub-national government units in most countries around the world have assumed some extent of decision-making authority (Cheema and Rondinelli 2007).[1] This often occurs through devolution, a reform that typically involves the transfer of different government functions related to sectors, such as health, from the central government to sub-national units. Devolution often entails changes to public financial management (PFM) rules, systems, and processes. In parallel, countries have also initiated health financing reforms, such as expanding publicly managed health insurance, eliminating user fees, and introducing performance-based payments to move closer to the goal of achieving universal health coverage (UHC). While national governments exercise a high degree of control over the design of health financing reforms, their implementation in devolved contexts is heavily influenced by local politics, policies, and processes. Sub-national government units are often the main purchasers of primary health care services, exercising considerable control over the funds flowing to public facilities, including from user fees or insurance reimbursements.

Public funds lie at the heart of sustainable health financing policy for achieving UHC (Kutzin, Yip, and Cashin 2016). Globally, public financing accounted for approximately 60% of health spending in 2017 and increased faster than any other source of health expenditure over the preceding decade (World Health Organization 2019). Given public funding's growing role, governments and development partners increasingly recognize the importance of PFM to effective, efficient, and equitable health spending (Cashin et al. 2017; Barroy et al. 2019). Therefore, devolution and related PFM reforms can affect how public funds are allocated, used, and reported in the health sector.

The World Health Organization (WHO) and ThinkWell jointly developed a series of case studies to explore the implications of devolution for health financing, with a deep dive into PFM issues. The cases shed light on how health financing functions are organized within and impacted by each country's devolved system of government. They also explore how devolution has shaped PFM processes in the health sector, including budget development, approval, execution, and accountability.

This case study details health financing and health-related PFM processes in Nigeria, a federation with nearly 70 years of experience with devolved governance. Since independence, Nigeria's political history has largely centered on competition for power and resources among national and sub-national actors (Elemo 2018; Owen 2018). Today, the federal, state, and local levels of government share a range of authorities and responsibilities, including with respect to health and health services. Together, these government tiers finance and manage an elaborate ecosystem of health agencies and service providers, which are both enabled and hindered by the federal structure in which they evolved.

METHODOLOGY

ThinkWell and WHO collected data in the selected countries that answer the following overarching questions:

- How are the three health financing functions—revenue raising, pooling, and purchasing—and related governance functions organized and affected by a devolved system of government in a country?
- What challenges related to devolved health financing exist and how do these affect progress toward UHC?

[1] The literature offers several typologies to distinguish between different forms of decentralization (devolution vs. de-concentration, administrative, fiscal vs. political decentralization), which are discussed in a separate methodology document.

- How do PFM processes unfold across government levels, and what is the role of sub-national governments in allocating, spending, and reporting public funds for health?
- What is the role of health facilities in PFM processes?

Data for Nigeria was mainly collected through desk review, supplemented with expert interviews. The desk review entailed a purposeful review of documents and data that could be accessed online or via a U.S.-based research library, including from federal and state governments, international organizations, development assistance projects, and peer-reviewed literature. It was not feasible to review information from all or most of Nigeria's 36 state governments, so when sources lacked state-specific information or examples, they were sought from two state governments (Lagos and Osun) whose websites contained substantial documentation. Information from other states was gathered more selectively. To address gaps, interviews were conducted with six experts on Nigeria's health financing system (**Annex 1**). Given Nigeria's complexity and diversity, the findings presented here are not intended to comprehensively capture all sub-national practices and policies. Instead, information from the state and local level illustrates some of the current realities, reform trajectories, and variability across the country. Notably, analysis of federal and state budget information occurred prior to the adoption of any budget amendments that reflect the fiscal impact of and public sector response to the COVID-19 pandemic.

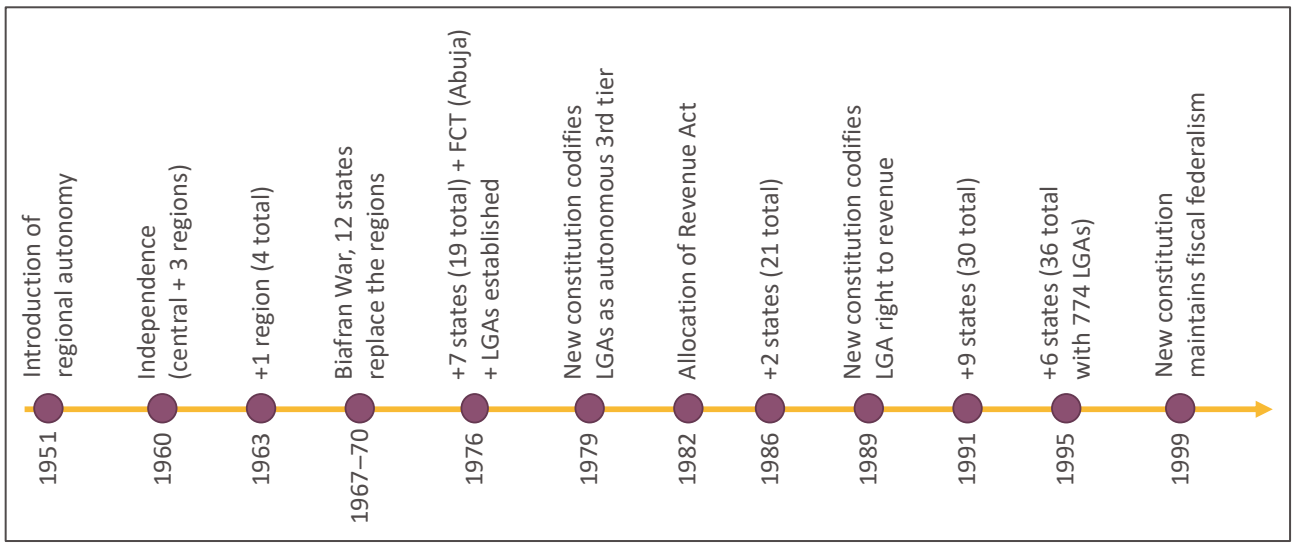
COUNTRY CONTEXT

Decentralization in Nigeria predates independence and every constitution since 1960 has included some form of devolved government (**Figure 1**)**.** The colonial constitution of 1951 introduced a degree of regional autonomy, which persisted until the late 1960s, when the Biafran War prompted the dissolution of the four regions into 12 less powerful states under a stronger central government. Seven additional states and the Federal Capital Territory (FCT) were created in 1976. In the same year, local government areas (LGAs) were established by decree as the third government tier; these were subsequently enshrined in the 1979 constitution. Devolved governance evolved considerably in the 1980s and 1990s. For instance, the 1982 Allocation of Revenue Act specified proportions of federal revenues for each governmental tier (federal, state, and local); two more states were created in 1986; the 1989 constitution required the National Assembly to allocate public revenue funds to local governments; fifteen additional states were created between 1991 and 1995; and the 1999 constitution maintained the three-tier system that prevails today, with power and resources distributed among Nigeria's federal government, FCT, 36 states, and 774 local governments areas (Umaroho 2006; Ogunyemi 2014; Fatile and Ejalonibu 2015; Kress, Su, and Wang 2016). Over time, state and local governments gradually accrued influence and authority and since 1999, they have played more assertive, albeit modest roles in policymaking and governance (Elemo 2018).

Most political and fiscal authority is vested either in the federal government or devolved to states, while local governments are subject to their states' willingness to delegate powers and financial autonomy. The 1999 Constitution specifies extensive exclusive and concurrent powers for the federal and state legislatures. In contrast, the functions of local government councils are modest and many fall under state government leadership, including "the provision and maintenance of health services" (Constitution of the Federal Republic of Nigeria 1999). Moreover, the federal government raised more than 92% of government revenue in 2019 (Central Bank of Nigeria 2020), but much of this is transferred. In 2016, state and local governments accounted for 36% and 11% of government spending, respectively. Additionally, government health spending was more concentrated at the federal level, with state and local governments accounting for 26% and 7%, respectively (Federal Ministry of Health 2017a). Despite local government rights to revenue, there is a history of states eroding the local tier's fiscal autonomy. Tactics have included diverting federal revenue intended for LGAs to state-defined priorities and operating parallel revenue boards that collect from sources intended for an LGA's revenue base (e.g., market fees). The constitutionality of these

state behaviors is subject to continual debate, and meanwhile, few LGAs have the resources or political influence to challenge their state governments (Adeyemo 2005; Okojie 2009; Fatile and Ejalonibu 2015; Elemo 2018; Expert Interviews 2020).

Figure 1. Selected events in the evolution of Nigeria's federal system



Sources: Umaroho 2006; Ogunyemi 2014; Fatile and Ejalonibu 2015; Kress, Su, and Wang 2016; Elemo 2018

Health sector responsibilities are both fragmented and overlapping across government tiers. Table 1 summarizes the distribution of levels of care and management responsibilities for public facilities across government tiers. In addition to policymaking, planning, coordination, and regulation, the Federal Ministry of Health (FMOH) oversees federal hospitals that deliver referral and specialized services. States are free to devise their own health policies and plans, and state ministries of health (SMOHs) manage their own hospitals that deliver curative care and basic specialist services (Hafez 2018; Alkenbrack et al. 2018). Local departments of health supervise primary health care (PHC) services, with state oversight. However, lines between the levels tend to blur. For example, PHC management, planning, and monitoring are driven more by states than local governments, and since 2011, the federal government has encouraged a Primary Health Care Under One Roof policy meant to further consolidate PHC governance under the authority of state-level PHC Boards (Health Partners International 2014; National Primary Health Care Development Agency 2015).[2] These PHC Boards are meant to be autonomous but often remain subordinate to SMOHs or act as extensions of the National Primary Health Care Development Agency (NPHCDA), a parastatal agency under the FMOH. Moreover, the federal government manages, oversees, and directly funds select programs that include PHC services—such as for HIV, immunizations, malaria, maternal and child health, and tuberculosis—rather than transferring all needed resources to state or local levels.[3] Local governments retain roles in oversight, resource planning, and responsiveness to citizen concerns (NPHCDA 2015; Eboreime et al. 2017; World Bank 2018c; Alkenbrack et al. 2018; Bawas 2019; Expert Interviews 2020).

[2] These are also to referred to as State PHC Development Boards or State PHC Development Agencies (SPHCDAs).

[3] The previously standalone National Program on Immunization was incorporated into the NPHCDA in 2007.

Table 1. Levels of care and facility types across government tiers

Tier	Level of care	Public facility types	How responsibilities blur across tiers
Federal	Tertiary	Teaching hospitals Other tertiary hospitals	FMOH and NPHCDA steer priority programs that include PHC services
State	Secondary	Specialist hospitals General hospitals	The Primary Health Care Under One Roof policy aims to bring management, planning, monitoring, and workforce under the authority of state-level PHC Boards, with local governments meant to oversee PHC facilities and responding to citizens. Implementation is ongoing and varies by state.
Local	Primary	Primary health centers Health clinics Health posts & dispensaries	

Source: Author, building on Figure 5 in Alkenbrack et al. 2018

There is an extensive PFM regime at the federal level, while state systems are much less developed. Foundational PFM legislation was adopted prior to independence, and in the 2000s several additional laws were adopted (see Box). The legal and institutional framework for PFM contains some overlaps in federal responsibilities. For example, the Federal Ministry of Finance (FMOF) is charged with fiscal management, while since 2015, the Ministry of Budget and National Planning has been responsible for budget management (World Bank 2018b).

Box: Institutional basis of the federal PFM regime.

The federal PFM regime comprises the 1999 constitution, the following laws and regulations, and additional Treasury circulars:

1956 Audit Act

1958 Finance (Control & Management) Act

1982 Allocation of Revenue Act

2004 Debt Management Office Act

2007 Public Procurement Act

2007 Fiscal Responsibility Act

2007 Federal Inland Revenue Services Act

2007 Central Bank of Nigeria Act

2008 Public Service Rules

2009 Financial Regulations

2019 Finance Act

Sources: Zubairu 2016; Inoykwe 2018; Expert Interviews 2020

HEALTH FINANCING LANDSCAPE IN THE CONTEXT OF DEVOLUTION

Nigeria has a complex, fragmented, and evolving health financing system. Figure 2 depicts the financial and in-kind resource flows from government, private, and donor sources to publicly managed service providers via an array of intermediate organizations. Each government tier contributes to funding and management of its facilities, drawing on its own revenue and resources received from higher levels and donors. Additionally, federal and state agencies, such as the NPHCDA and corresponding state PHC Boards, provide inputs, oversight, and technical support to the PHC level. Historically, health insurance has played a minimal role: the National Health Insurance Scheme (NHIS) covers less than 5% of the population, and enrollment is modest in the many contributory State Social Health Insurance Schemes (SSHIS). The new

Basic Health Care Provision Fund (BHCPF) aims to bolster PHC service quality and access by channeling more funds to frontline facilities via both supply-side investments (managed by PHC Boards) and demand-side reimbursement (from pools managed by SSHIS) (Hafez 2018; Expert Interviews 2020).

The sub-sections that follow detail how—across government tiers—funds are collected and shared (**revenue raising**), managed on behalf of the population (**pooling**), and used to pay for health services (**purchasing**). A final sub-section examines **reporting, oversight, and accountability** for health financing. Findings related to **PFM processes** are woven throughout.

REVENUE RAISING

Revenue raising refers to the way funds are mobilized for health care provision and promotion activities. This section explores revenue raising at the federal, state, and local levels. The analysis refers to several key health financing indicators, which are summarized in **Table 2** below.

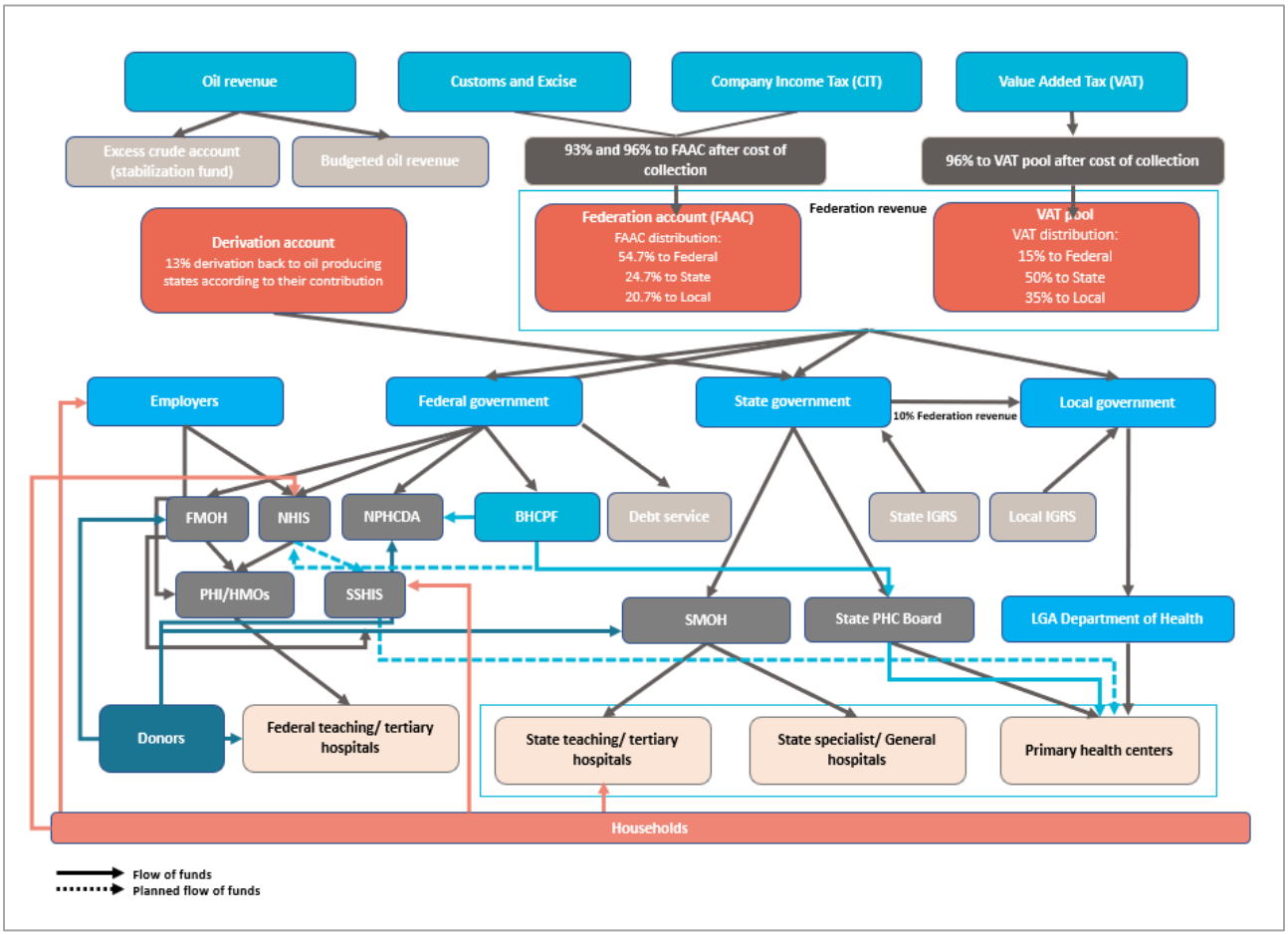
Table 2. Summary of health financing indicators

Indicator	Value (2016)
Gross government expenditure as a share of gross domestic product (GGE / GDP)	9.4%
Total health expenditure as a share of GDP (THE / GDP)	3.8%
Gross government health expenditure as a share of gross government expenditure (GGHE / GGE)	5.1%
Federal	*5.9%*
State	*4.2%*
Local	*3.8%*
Gross government health expenditure as a share of current health expenditure (GGHE / CHE)	13.1%
Out-of-pocket household health expenditure as a share of current health expenditure (OOP / CHE)	75.2%
Donor health expenditure as a share of current health expenditure (EXT / CHE)	10.3%

Source: FMOH 2017a

Note: These data, for 2016, are from the government's most recent published health accounts, which provides sufficient detail to disaggregate spending across the three government tiers. Data for 2017 and 2018 in the Global Health Expenditure Database indicate slight increases in the government and OOP shares of CHE, concurrent to a small decrease in the donor share.

Figure 2. Diagram of health system funds flows



Source: Reproduction (with permission) of Figure 1.8 in Hafez 2018, with author's updates

Abbreviations: BHCPF = Basic Health Care Provision Fund; F/SMOH = Federal/State Ministry of Health; IGR = internally generated revenue; LGA = local government area; NPHCDA = National Primary Health Care Development Agency; N/SSHIS = National/State Social Health Insurance Schemes

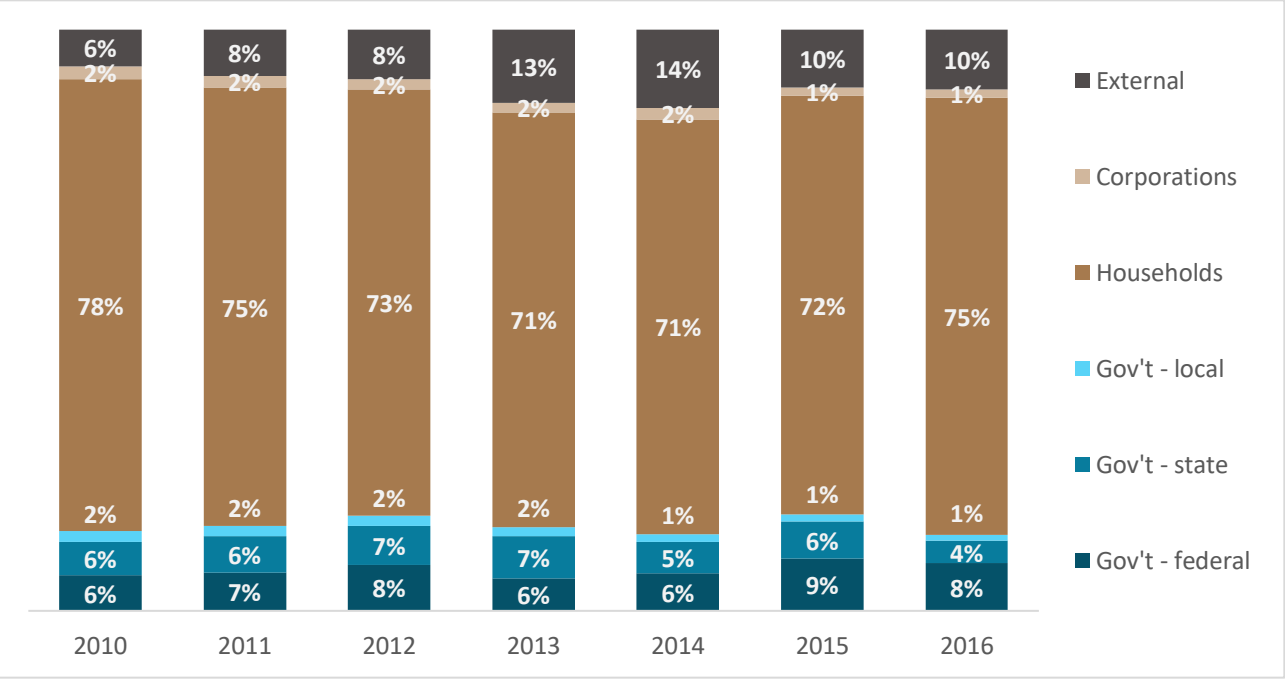
Revenue Collection

The health sector is underfinanced, and there in minimal financial protection. In 2016, the health sector claimed just 5% of gross government expenditure (GGE), and public sources only accounted for 13.1% of current health expenditure (CHE). In real terms, per capita government spending on health has been roughly constant or declining since 2006.[4] Meanwhile, households' out-of-pocket (OOP) spending has long been the predominant source of health system revenue, accounting for around three-quarters of health expenditure between 2010 and 2016 (**Figure 3**). This suggests that most Nigerians are not well protected from the costs of health services, consistent with the reality that only 4.2% of the population was enrolled in the NHIS as of 2016 and that, to date, membership has remained limited across the numerous SSHIS. In principle, these schemes are open to all formal sector workers, though most NHIS members are federal civil servants and their dependents (Hafez 2018; Expert Interviews 2020). In total, SSHIS accounted for only 1%

[4] Data from the government's health accounts publications (FMOH 2010; 2017a) indicate a small increase (2%) between 2006 and 2016, while data in the Global Health Expenditure Database (WHO 2020) indicate a moderate decrease (12%) between 2006 and 2018, with considerable volatility during the intervening years.

of CHE in 2016 (WHO 2020). The government provided the next largest share of health spending during 2010–2016, when the three tiers collectively accounted for a range of 12% to 17%. The federal tier's share of expenditures grew from 6% in 2010 to 8% in 2016, while the state and local shares decreased from 6% to 4% and 2% to 1%, respectively. Aside from OOP spending, domestic private sources, such as corporations, accounted for 2% or less of spending. Finally, external sources contributed between 6% and 14% each year.

Figure 3. Composition of health expenditure by source, 2010–2016



Source: FMOH 2017a

Externally sourced funds support investments in both health system strengthening and select health programs. Leading contributors include Gavi, the Vaccine Alliance; the Global Fund to Fight AIDS, Tuberculosis, and Malaria; the United States government; and the World Bank. Together, they blend on-budget, off-budget, and in-kind support to priority programs, including those for family planning, HIV, immunization, malaria, neglected tropical diseases, and tuberculosis. Donors direct support across government tiers. For example, as part of the immunization program, Gavi subsidizes vaccines that are procured by UNICEF and then distributed by the NPHCDA to sub-national levels, where Gavi and other partners also help to pay health worker per diems. Many of these priority programs are highly donor dependent. For instance, external sources covered two-thirds of routine immunization spending between 2014 and 2016 and more than four-fifths of HIV spending in 2018. Similarly, donors consistently support family planning services while government funding has been volatile in recent years (Alkenbrack et al. 2018; Hafez 2018; World Bank 2018c; Advance Family Planning 2019; National Assembly 2020; U.S. President's Emergency Plan for AIDS Relief 2020).

All three government tiers raise revenue, with the federal government collecting significantly more than the others. Legislation defines how each tier may raise revenue, with the most lucrative sources—oil and company income taxes—assigned to the federal level. States collect 7% of public revenue, mainly via personal income taxes, and local governments generate only 0.3% of revenue through instruments like license, market, and utility fees (Hafez 2018; Udoma 2019; Central Bank of Nigeria 2020; National Bureau of Statistics 2020).

Intergovernmental Transfers

Federal revenue is subject to strict vertical sharing rules. For decades, the contentious dynamics between the federal and state governments have centered largely on access to revenue from agriculture, oil, and other sources. Today, rights to federally collected oil, value-added tax (VAT), and other revenues (pooled in the Federation Account[5]) are defined by the Allocation of Revenue Act of 1982 and subsequent recommendations provided by the Revenue Mobilisation Allocation and Fiscal Commission (Ogunyemi 2014; Elemo 2018). The Federation Account Allocation Committee meets monthly to review the previous month's revenue and distribute it across the three government tiers ("Revenue Allocation" n.d.). The current vertical sharing rules are as follows (Hafez 2018):[6]

- **Oil revenue**: 13% to oil-producing states in proportion to their contributions, 87% to the Federation Account;
- **VAT revenue**: 15% to the federal government, 50% to states, and 35% to local governments; and
- **Federation Account**: 54.7% to the federal government, 24.7% to states, and 20.7% to local governments.

States and local governments depend mainly on transfers from higher government tiers, supplemented by their own revenue collection. In 2019, federal transfers accounted for 65% of state revenue and 60% of state expenditure.[7] At the same time, 95% of LGA revenue came from federal transfers and state allocations. Reliance on intergovernmental transfers varied widely across states, from 23% of revenue in Lagos to almost 90% in Bayelsa. Only two of 36 states, Lagos and Ogun, generated more revenue internally than they received through federal transfer (Central Bank of Nigeria 2020; National Bureau of Statistics 2020). The transfers are unconditional block grants to state ministries of finance, meaning that state governments have full autonomy to pool, allocate, and manage transferred funds together with their internally generated revenue (Kress, Su, and Wang 2016; Hafez 2018; Alkenbrack et al. 2018). Declining oil prices in the last decade have re-focused both federal and state attention on diversifying revenue sources and generating more revenue at the sub-national level, with varying success (Owen 2018).

In practice, local governments are at the mercy of states when it comes to revenue. Federal transfers for local governments flow into state-controlled Joint Accounts, giving state governments considerable control over their disbursement and use. By law, states are also supposed to contribute 10% of their internally generated revenue (IGR) to the Joint Accounts, though implementation is inconsistent (Fatile and Ejalonibu 2015; Inoykwe 2018). There is continual advocacy from the local level to eliminate Joint Accounts and allow revenue to flow directly to local government authorities, with limited success. In practice, local governments wield minimal political power in most states, and state governments withhold a substantial portion of locally bound revenue. There is ongoing constitutional and political debate regarding the extent to which states are within their rights to fully control resource flows to the local level (Elemo 2018; Expert Interviews 2020).

The horizontal revenue sharing rules, which govern allocations of federal transfers across state and local governments, do more to exacerbate inequity than to address it. First, they favor wealthier, oil-producing states. Oil-producing states receive allocations of oil revenue twice, first based on derivation and then from distribution of the Federation Account. **Table 3** details how Federation Account and VAT revenues are shared among states. Second, among the parameters for horizontal distribution of the Federation Account,

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[5] The Federation Account is one of two main accounts into which federal revenue accrues. It holds oil and other non-VAT revenue, which is then distributed across government tiers according to the vertical revenue sharing rules.

[6] These proportions are determined after debiting collection costs.

[7] The discrepancy between revenue and expenditure is explained by debt-financed deficit spending.

equality among the states is the most prominent: more than 45% of Federation Account revenues are divided equally among the states (i.e., each state receives the same absolute amount), while smaller portions are distributed based on factors with the potential to enhance equity, including population (26%), health (3%), education (3%), potable water (1.5%), and population density (1.5%). Similarly, 50% of VAT revenue is distributed based on derivation, 40% is distributed equally among the states, and only 10% is distributed by population size. Third, the formula rewards IGR, again favoring economically stronger states. More than 8% of Federation Account revenue is divided among states in direct proportion to how much internal revenue they generate. Finally, the formula's health parameter is based on numbers of hospital beds rather than health needs or outputs, which rewards states that can already afford more health infrastructure (Elemo 2018; Hafez 2018). Based on the Federation Account allocation formula, each state receives a pot of revenue meant for local government use, which states can supplement with their own resources. However, in practice, states control how much revenue each local government receives, often allowing little funding to reach the local level (Inoykwe 2018; Expert Interviews 2020). Taken together, the revenue sharing rules in Nigeria's devolved system reinforce the inequitable distribution of resources across states (**Figure 4**).

Increasingly, states are implementing their own state social health insurance schemes (SSHIS) to collect revenue and improve financial protection. These schemes are financial and operationally separate from the NHIS. Under the National Health Act of 2014, a state must establish a PHC Board and SSHIS to access Basic Health Care Provision Fund resources. A 2015 memorandum issued by the National Council on Health gave further impetus to SSHIS by calling for the decentralization of publicly managed health insurance due to consistently low enrollment in the NHIS. By 2020, 27 states and the FCT had passed an SSHIS Law (Hafez 2018; Ogundeji, Ohiri, and Agidani 2019; DGI Consult 2020; Expert Interviews 2020). The typical revenue model for SSHIS include payroll taxes or another direct contribution, a state-financed equity trust or fund, and an allocation from the state's health budget for the scheme's operating expenses. For example, Cross River state's 2016 health insurance law requires all state residents to pay at least NGN 1,000 per month.[8] The state also earmarked 1% of its consolidated revenue fund for an equity fund to cover vulnerable populations, and allocated NGN 150 million to seed the health insurance fund (Channels Television 2016; Health Finance and Governance Project 2016).[9] Following several years of preparation, a formal launch in 2018, and efforts to improve service availability and quality, the state planned to enroll residents in the scheme starting in October 2020 (Edem 2018; The Sun Nigeria 2020). Schemes have already enrolled member in 16 other states, including 10 where enrollees are now accessing covered services (DGI Consult 2020).

Table 3. Allocation factors for sharing Federation Account and VAT revenues among states

Allocation factor	Percentage of Federation Account revenues	Percentage of VAT revenues
Equal amount to each state	45.23	40.00
Population	25.60	10.00
Population density	1.45	--
Internal revenue effort	8.31	--

[8] Equivalent to almost US$3 in 2020.

[9] Equivalent to nearly US$400,000 in 2020.

Allocation factor	Percentage of Federation Account revenues	Percentage of VAT revenues
Landmass (share of national territory)	5.35	--
Terrain	5.35	--
Rural roads and inland waterways	1.21	--
Potable water	1.50	--
Education	3.00	--
Health	3.00	--
Derivation	--	50.00

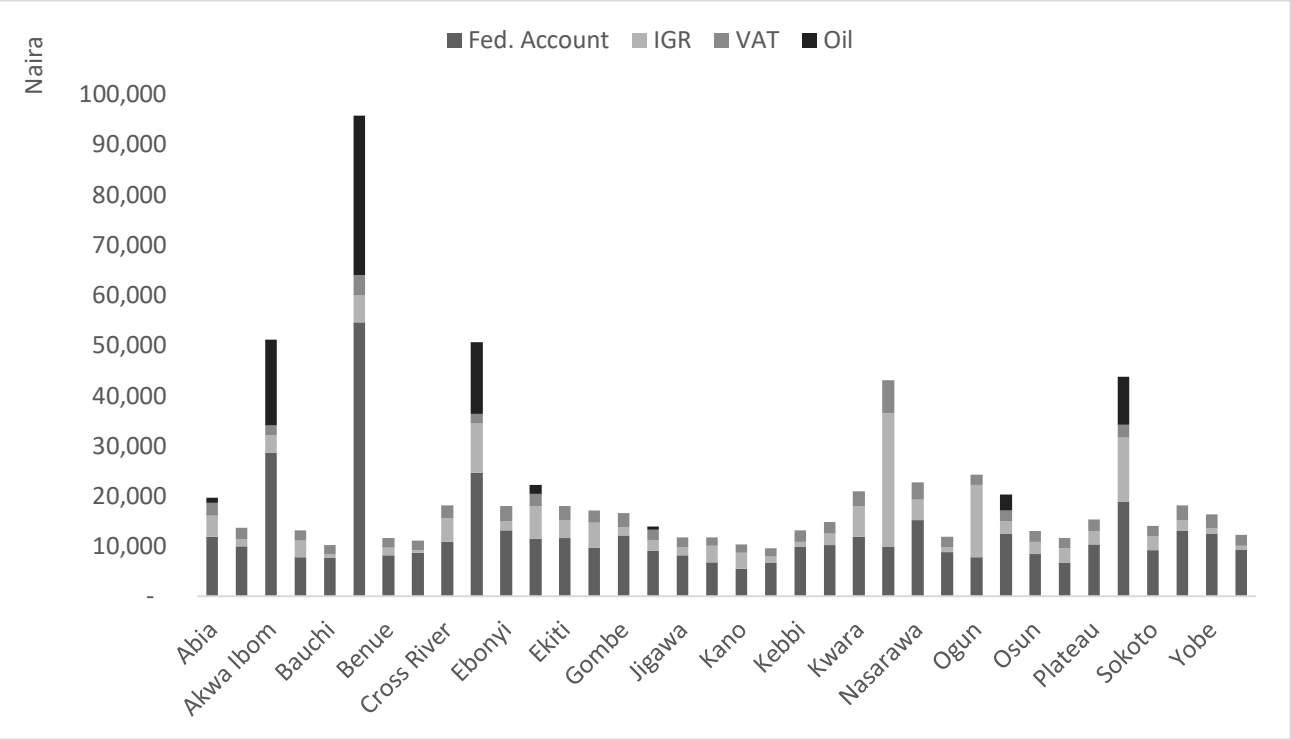
Source: Table 1.7 in Hafez 2018

The BHCPF provides an additional way to channel federal revenue to PHC facilities. Per the National Health Act of 2014, at least 1% of federal revenue is meant to be earmarked for the BHCPF, which is meant to help ensure all Nigerians can access a basic package of care, beginning with the poor (Uzochukwu, Onwujekwe, and Mbachu 2015). The Act also requires 25% counterpart funding from the states, which remains contentious. Funds are meant to flow through three gateways (Expert Interviews 2020; NPHCDA, NHIS, and NEMTC 2020), including:

1. A demand-side NHIS gateway (50% of the BHCPF), through which funds pass to the NHIS and then BHCPF-specific accounts managed by SSHIS that will be used to buy services from contracted PHC providers. These funds are intended to first cover the poorest segments of each state's population, while funds will be managed separately for enrollees (primarily state civil servants and other formal sector workers) in the other state schemes;
2. A supply-side NPHCDA gateway (45%), through which funds will pass to the NPHCDA and then to state PHC Boards, which will transfer money to selected PHC facilities to cover operating costs. These funds will be additional to the allocations states already make to their PHC Boards via their health budgets; and
3. An Emergency Medical Treatment (EMT) gateway (5%).

Transfers to states from the BHCPF are separate from the main intergovernmental transfers, which flow to state ministries of finance in the form of unconditional block grants. The BHCPF is a portion of the revenue retained at the federal level that is then separately transferred to the states specifically for health.

Figure 4. State revenue per capita from federal and internal sources, 2017



Source: Uche et al. 2019
Abbreviations: Fed. = Federation; IGR = internally generated revenue; VAT = value-added tax

Preparations for implementing the BHCPF are ongoing. Despite federal allocations starting in 2019, these have not yet met the statutory requirement that at least 1% of federal revenue be earmarked for the Fund. A line item was introduced in 2019 for NGN 51 billion (0.7% of federal revenue),[10] followed by a statutory transfer to the Fund of NGN 44 billion (0.5% of federal revenue) in 2010[11] (National Assembly 2019; 2020; Okeowo et al. 2019; Uche et al. 2020). Additionally, the Global Financing Facility for Every Woman Every Child (GFF) and World Bank-implemented multi-donor trust fund committed US$20 million to support the BHCPF at the federal level and in three states during 2018–2022,[12] including US$17 million to directly finance service delivery and US$3 million to strengthen management systems for BHCPF implementation (GFF 2018). Notably, BHCPF operations were suspended in 2019 due to disputes over the fund's design, leading to extensive stakeholder engagement by the National Council on Health, which approved new operating guidelines in August 2020. At that time, fund flows via the supply-side gateway restarted. The demand-side and EMT gateways are not yet operational (Expert Interviews 2020).

Budget Development and Negotiation

The budget development process is similar across tiers, but there is minimal coordination between the federal and state tiers, with the latter dictating local practices. Table 4 summarizes key budget development and approval roles across the government tiers. Federal and state executives propose budgets that are debated, modified, and approved by their respective legislatures, which then appropriate

[10] Equivalent to roughly US$142 million.
[11] Equivalent to roughly US$115 million.
[12] The participating states are Abia, Niger, and Osun.

the funds. State and local governments, on the other hand, rely on the federal government's revenue projections to formulate their budgets (FMOH 2017c). Constitutionally, states control the local budgeting process and determine how autonomously—or not—it is conducted (Fatile and Ejalonibu 2015). In some states, local government councils approve local budgets, while in others, the state houses of assembly manage this approval (World Bank 2018c).

At all three levels, budget development begins when finance authorities issue calls for proposals to all government ministries, departments, and agencies (MDAs) during the previous year.

Table 5. Key dates for development of the 2021 federal budget

April 23, 2020	Personnel Cost Budget Call Circular Issued
July 21, 2020	MTEF/FSP Published
August 7, 2020	Non-Personnel Budget Call Circular Issued
August 7-14, 2020	GIFMIS refresher training
August 14, 2020	Deadline for GOE budgets
August 28, 2020	Deadline for MDA budgets
September 7-15, 2020	Budget Bilateral Meetings
September 2020	Submission to National Assembly
December 2020	Adoption of Appropriation Bill

Sources: Olisah 2020; Budget Office of the Federation 2020a, 2020b, 2020c, 2020d

Abbreviations: GFMIS = General Integrated Financial Management Information System; GOE = Government-owned Enterprise; MDA = ministry, department, and agency; MTEF/FSP = Medium Term Expenditure Framework and Fiscal Strategy Paper

contains key dates for development of the 2021 federal budget; states broadly mirror this timeline. Following the budget calls, MDAs collect input from all subsidiary agencies and some hold stakeholder consultations, including with traditional and community leaders at the local level. Federal MDAs use a budget preparation module in the Government Integrated Financial Management Information System (GIFMIS), a computerized system for budgeting and accounting. Some states also deploy web-based budgeting platforms, while others rely on more manual processes, such as Microsoft Excel-based budget tracking (Ministry of Budget and National Planning 2018; World Bank 2018c).

Table 4. Key budget development and approval roles

Tier	Proposal lead	Approval
Federal	Ministry of Finance, Budget, and National Planning	National Assembly
State	Ministries of Finance	State Houses of Assembly
Local	Executive	Local government councils or State Houses of Assembly

Sources: Fatile and Ejalonibu 2015, FMOH 2017c

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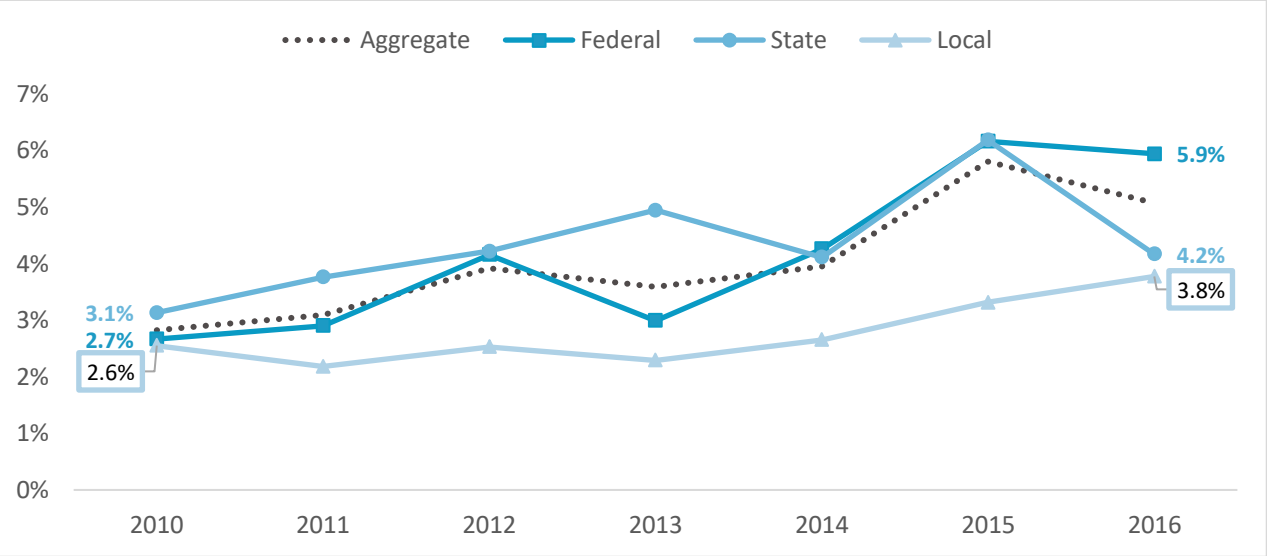
Abbreviations: GFMIS = General Integrated Financial Management Information System; GOE = Government-owned Enterprise; MDA = ministry, department, and agency; MTEF/FSP = Medium Term Expenditure Framework and Fiscal Strategy Paper

Budget structures vary across government tiers and states, and in general "budgets are not prepared according international definitions and standards" (World Bank 2018c, 6). The federal budget is structured primarily by administrative and economic classifications (**Annex 2**). The overall budget is organized by the FMOH and its budget is further sub-divided between the ministry itself and nearly 120 other budget units under its purview (e.g., NPCHDA, National Blood Transfusion Services, each federal health facility, and more). Each unit's budget is broken down into economic classifications, and the "Capital Expenditure" section is also presented by each investment project (National Assembly 2018; 2019; 2020). Implementation of program- and zero-based budgeting is ongoing. In recent years, the FMOH has been lagging behind other federal MDAs in terms of methodology, capacity, and preparation for budget discussions (Alkenbrack et al. 2018). Budget standards vary across states and broad budget categories, such as personnel, operations, and capital investment, offer little detail and are applied inconsistently (World Bank 2018c). For example, the Lagos state budget (**Annex 3**) is functionally organized and then sub-divided by sector MDAs (e.g., state MOH and state-managed facilities). Some MDAs are further divided into individual lines for both administrative units and capital projects. Each budget line is disaggregated into columns for personnel, overhead, and capital spending (Lagos State House of Assembly 2020). The Osun state budget contains similar information but presents each MDA's capital, personnel, and non-recurrent budget details separately (**Annex 4**). Unlike in the Lagos budget, Osun's administrative codes appear to mirror those in the federal budget (State Government of Osun 2020).

Efforts are ongoing to link health budgeting to policy priorities and fiscal constraints more rigorously. The federal government articulates fiscal priorities, among others, in its medium-term expenditure framework (MTEF) and medium-term sector strategies (MTSS). However, statements of policy priorities are not reliably translated into budget allocations to or within the FMOH, and there is no sector- or MDA-specific MTEF. Additionally, planning and budgeting are housed in separate departments. The FMOH's Department of Health Planning lacks sufficient resources —for instance, staff lack computers, internet connectivity, and official email accounts—while the Finance and Accounts Department views budgeting as no more than an accounting exercise. Meanwhile, state practices vary; in some states (e.g., Ekiti) the government develops

an MTEF and MTSS, while elsewhere (e.g., Niger) it does not. Either way, there is generally a disconnect between bottom-up budgeting and the sector budgets submitted to state planning authorities. The same is true at the local level, where even if PHC facilities have annual operating plans, they do not participate in broader budget formulation processes. Finally, unrealistic revenue projections hamper efficient budget allocation and implementation across all levels, and the data required for preparing good health budgets, including on coverage, utilization, wastage, and costs, are not reliably available (Alkenbrack et al. 2018; Hafez 2018; World Bank 2018c; Expert Interviews 2020).

Figure 5. Health's share of public expenditure in aggregate and by government tier, 2010–2016



Source: FMOH 2017a

Prioritization of health within government spending is low overall but greater at the federal than the state and local levels. In total, the health sector's share of GGE rose modestly from less than 3% in 2010 to around 5% in 2016 (FMOH 2017a; WHO 2020). In 2010, the three tiers allocated comparable shares of their spending to health, but by 2016, the federal government devoted more of its resources to health than the others (**Figure 5**). More recent budget analyses suggest similar levels of prioritization. For example, only 4.2% of the 2020 federal budget is allocated to health (Uche et al. 2020), and in 2019, states collectively allocated 7.4% of their budgets to the health sector, ranging from 2.2% in Bayelsa to 17.8% in Bwara (Uche et al. 2019). However, it is unclear if these budget share estimates fully capture health-related allocations to the local level or in other sectors. For example, in its Governance and Administration sector, Osun's state budget includes a line for the Local Governments Service Commission (**Figure 6**), which pays for at least a portion of the PHC workforce (alongside those employed in other local government services). The Lagos state budget makes similar provisions (Lagos State House of Assembly 2020). Additionally, the employer's share of civil servants' NHIS and SSHIS premiums is budgeted differently by federal and state governments. In the federal budget, each MDA's personnel costs include a line for NHIS contributions (see **Figure 7** for an example from the Federal Ministry of Information and Culture). Health insurance premiums for Lagos states public servants appear in the State Ministry of Establishment and Training's budget (Lagos State House of Assembly 2020), while in Osun, the State Ministry of Finance's budget has a line for NHIS and Osun Health Insurance Scheme contributions (State Government of Osun 2020).

Figure 6. Allocation to the Local Governments Service Commission in the 2020 Osun state budget

STATE GOVERNMENT OF OSUN

2020 ESTIMATES

OVERALL BUDGET SUMMARY

ADMINISTRATIVE CODES	MDAS	10000000 REVENUE	40000000 CAPITAL RECEIPTS (LOANS / BORROWINGS / EQUITY)	TOTAL INFLOW BUDGET	21000000 PERSONNEL	22000000 OTHER NON-DEBT RECURRENT	DEBT REPAYMENT
		₦	₦	₦	₦	₦	₦
	GOVERNANCE AND ADMINISTRATION SECTOR						
011100100100	OFFICE OF THE GOVERNOR	350,000,000.00	-	350,000,000.00	597,793,240.00	4,407,059,870.00	-
011100100900	OFFICE OF POLICY COORDINATION	-	-	-	-	-	-
011101000100	PUBLIC PROCUREMENT AGENCY	8,000,000.00	-	8,000,000.00	16,210,340.00	33,460,000.00	-
011101900100	MINISTRY OF SPECIAL DUTIES	1,000,000.00	-	1,000,000.00	3,943,680.00	10,575,820.00	-
011103800100	BUREAU OF PUBLIC SERVICE PENSION	10,000,000.00	-	10,000,000.00	9,031,543,220.00	2,643,960.00	-
011200300100	OSUN STATE HOUSE OF ASSEMBLY	2,200,000.00	-	2,200,000.00	423,785,650.00	1,290,503,940.00	-
011200400100	OSUN STATE HOUSE OF ASSEMBLY SERVICE COMMISSION	250,000.00	-	250,000.00	64,119,790.00	69,230,000.00	-
014000100100	OFFICE OF THE AUDITOR GENERAL (STATE)	2,000,000.00	-	2,000,000.00	113,965,290.00	12,172,990.00	-
014000200100	OFFICE OF THE AUDITOR GENERAL (LOCAL GOVERNMENTS)	435,700,000.00	-	435,700,000.00	68,733,860.00	12,690,990.00	-
012500600100	MINISTRY OF HUMAN RESOURCES & CAPACITY BUILDING	5,200,000.00	-	5,200,000.00	73,196,190.00	211,542,310.00	-
014700100100	CIVIL SERVICE COMMISSION	5,000,000.00	-	5,000,000.00	68,104,290.00	15,000,000.00	-
014700200100	LOCAL GOVERNMENTS SERVICE COMMISSION	407,936,500.00	-	407,936,500.00	44,384,810.00	400,000,000.00	-
014800100100	OSUN STATE INDEPENDENT ELECTORAL COMMISSION	10,000,000.00	-	10,000,000.00	70,584,550.00	1,500,000,000.00	-
022800100100	MINISTRY OF INNOVATION, SCIENCE AND TECHNOLOGY	20,000,000.00	-	20,000,000.00	72,778,650.00	32,358,000.00	-
045102100100	MINISTRY OF REGIONAL INTEGRATION	500,000.00	-	500,000.00	-	46,000,000.00	-
085100100100	MINISTRY OF LOCAL GOVERNMENTS AND CHIEFTAINCY AFFAIRS	50,000,000.00	-	50,000,000.00	59,512,360.00	12,990,000.00	-
	SUB-TOTAL GOVERNANCE AND ADMINISTRATION SECTOR	**1,307,786,500.00**	-	**1,307,786,500.00**	**10,705,655,920.00**	**8,056,224,880.00**	-
	AGRICULTURE SECTOR						

Source: State Government of Osun 2020

Figure 7. Allocation for NHIS in the Federal Ministry of Information and Culture's 2019 budget

Federal Republic of Nigeria 2019 APPROPRIATION BILL

0123001001	FEDERAL MINISTRY OF INFORMATION & CULTURE - HQTRS		2019 APPROPRIATION ACT
CODE	LINE ITEM		AMOUNT
2101	**SALARY**		**143,115,773**
210101	**SALARIES AND WAGES**		**143,115,773**
21010101	SALARY		143,115,773
2102	**ALLOWANCES AND SOCIAL CONTRIBUTION**		**36,738,846**
210201	**ALLOWANCES**		**18,849,374**
21020101	NON REGULAR ALLOWANCES		18,849,374
210202	**SOCIAL CONTRIBUTIONS**		**17,889,472**
21020201	NHIS		7,155,789
21020202	CONTRIBUTORY PENSION - EMPLOYER'S CONTRIBUTION		10,733,683
22	**OTHER RECURRENT COSTS**		**83,909,990**

Source: National Assembly 2019

Abbreviation: NHIS = National Health Insurance Scheme

Although each government tier determines its own priorities within health, all three levels are constrained by large wage bills. Health allocations are hardly strategic; rather, resources largely follow health infrastructure and the distribution of care responsibilities, which skews spending toward hospital-based curative care and away from lower-cost, higher-impact public health and PHC services (Hafez 2018). Additionally, personnel accounts for substantial shares of 2020 health budgets across all levels, including 86% at the federal level (National Assembly 2020), 56% in Lagos (Lagos State House of Assembly 2020), and 91% in Osun (State Government of Osun 2020). It is difficult to discern from the budgets alone whether Lagos has prioritized budget space for other costs. For example, it appears that the Lagos budget more systematically includes projected, recurrent non-personnel spending financed by hospital-generated revenue. There are fewer available data regarding the composition of local health spending, which also appears to be skewed to human resources. The Lagos and Osun budgets include provisions for their Local Government Service Commissions, but no other allocations that are identifiably for recurrent spending at the local level. Moreover, several sources report that local governments rarely receive or allocate sufficient

funds for anything beyond personnel in the social sectors (Fatile and Ejalonibu 2015; Alkenbrack et al. 2018; Hafez 2018), and, as noted above, a majority of surveyed primary health centers report a lack of receipt of fund allocations for operating expenses from any government source (FMOH 2017b). Finally, there are unpublished analyses showing that one LGA in Kaduna State spent 95% of its recurrent health budget on employee salaries.[13]

Despite responsibility for overseeing PHC services, local governments are the least important tier with respect to PHC funding. As noted above, the local tier accounts for only 7% of overall government health expenditure (FMOH 2017a), reflecting the general misalignment between the resources available to local governments and their supposed expenditure responsibilities in health(Adeyemo 2005; Fatile and Ejalonibu 2015). Moreover, states have, in many ways, subjugated local governments for decades, for example by controlling their revenue via Joint Accounts. The Primary Health Care Under One Roof policy acknowledges this reality and concentrates financing and management responsibilities under state PHC Boards. This has entailed significant cuts to already meager local government budgets because the employment of lower-level health worker teams has shifted from local governments to the PHC Board in many states. State-local dynamics remain fluid, and it is unclear if increased, unconditional funding under local government control would translate into more resources for PHC. Historically, health has not fared well in local government priority setting (Expert Interviews 2020).

The federal government cannot impose sector minimums on sub-national budgets (World Bank 2018c). However, the federal budget does include resources for coordination and in-kind support for vertical programs, which facilitates inclusion of federal and donor priorities in state and local services. In their capital budgets, the FMOH and NPHCDA list one or more projects for health areas such as cancer control, family planning, HIV, immunization, malaria, neglected tropical diseases, nutrition, and tuberculosis (**Figure 8**). Some of these are explicitly allocated as donor counterpart financing (**Figure 9**).

Figure 8. Allocations for immunization program activities in the 2020 NPHCDA capital budget

0521003001	NATIONAL PRIMARY HEALTH CARE DEVELOPMENT AGENCY		
CODE	**PROJECT NAME**	**TYPE**	**AMOUNT**
ERGP23126890	RENOVATION & EQUIPPING OF GAWASA PHC FACILITY IN BANDAN WARD SURU LGA KEBBI STATE	ONGOING	16,683,587
ERGP25112666	PROCUREMENT OF RI VACCINES, DEVICES AND OPERATIONAL COST	ONGOING	533,584,491
ERGP25112672	POLIO ERADICATION INITIATIVE (PEI)	ONGOING	4,800,000,000
ERGP25112677	PROCUREMENT OF NON POLIO SIA VACCINE, DEVICE AND OPERATIONAL COST	ONGOING	615,206,326
ERGP25112679	PROCUREMENT OF OUTBREAK (EMERGENCY) RESPONSE VACCINE, DEVICES AND OPERATIONAL COST	ONGOING	1,028,363,525
ERGP25112686	DISEASE CONTROL AND OPERATIONAL COST FOR SURVELLANCE ACTIVITIES	ONGOING	77,563,500
ERGP25112706	MONITORING AND EVALUATION OF PHC SYSTEMS AND SERVICES	ONGOING	52,239,500
ERGP25112708	QUARTERLY BUDGET MONITORING AND EXPENDITURE TRACKING	ONGOING	44,294,120
ERGP25112722	PHC PROJECT PLANNING,REVIEW AND OPERATIONAL RESEARCH	ONGOING	11,651,150
ERGP25126379	PROCUREMENT OF TRAVELERS AND PILGRIMS VACCINES	ONGOING	271,804,444
ERGP25126400	VACCINE DISTRIBUTION AND TRANSPORT (NATIONAL TO STATES & HEALTH FACILITIES)	ONGOING	319,660,200
ERGP25126416	VACCINE SUPPLY CHAIN GOVERNANCE	ONGOING	76,942,400
ERGP25126430	IMMUNIZATION SUPPLY CHAIN STRENGTHENING AT NSCS AND ZONAL COLD STORES	ONGOING	81,188,000

Source: National Assembly 2020
Abbreviation: NSCS = National Strategic Cold Store; RI = routine immunization

[13] The unpublished findings are described in Kress, Su and Wang 2016.

Figure 9. Allocation for family planning program counterpart funding in the 2020 FMOH budget

0521001001	FEDERAL MINISTRY OF HEALTH - HQTRS		
CODE	PROJECT NAME	TYPE	AMOUNT
ERGP25112202	IMNCH STRATEGY - ARTICULATION OF POLICY AND STRATEGY PLANS OF NEWBORN IN CHILD HEALTH IN LINE WITH SDG TARGETS ON INTEGRATED MANAGEMENT OF CHILDHOOD ILLNESSES (IMCI) FOLLOW UP, INTEGRATED SUPPORTIVE SUPERVISION (ISS) AND KANGAROO MOTHER CARE (KMC)	ONGOING	235,057,243
ERGP25112207	PROCUREMENT AND DISTRIBUTION OF FAMILY PLANNING COMMODITIES THROUGH COUNTERPART FUNDING TO UNFP	ONGOING	1,200,000,000

Source: National Assembly 2020
Abbreviation: UNFP = United Nations Population Fund

POOLING

Pooling refers to arrangements—including location, mechanism, and organization—for accumulating pre-paid funds that are raised for health. In Nigeria, funds are pooled in several ways across government tiers.

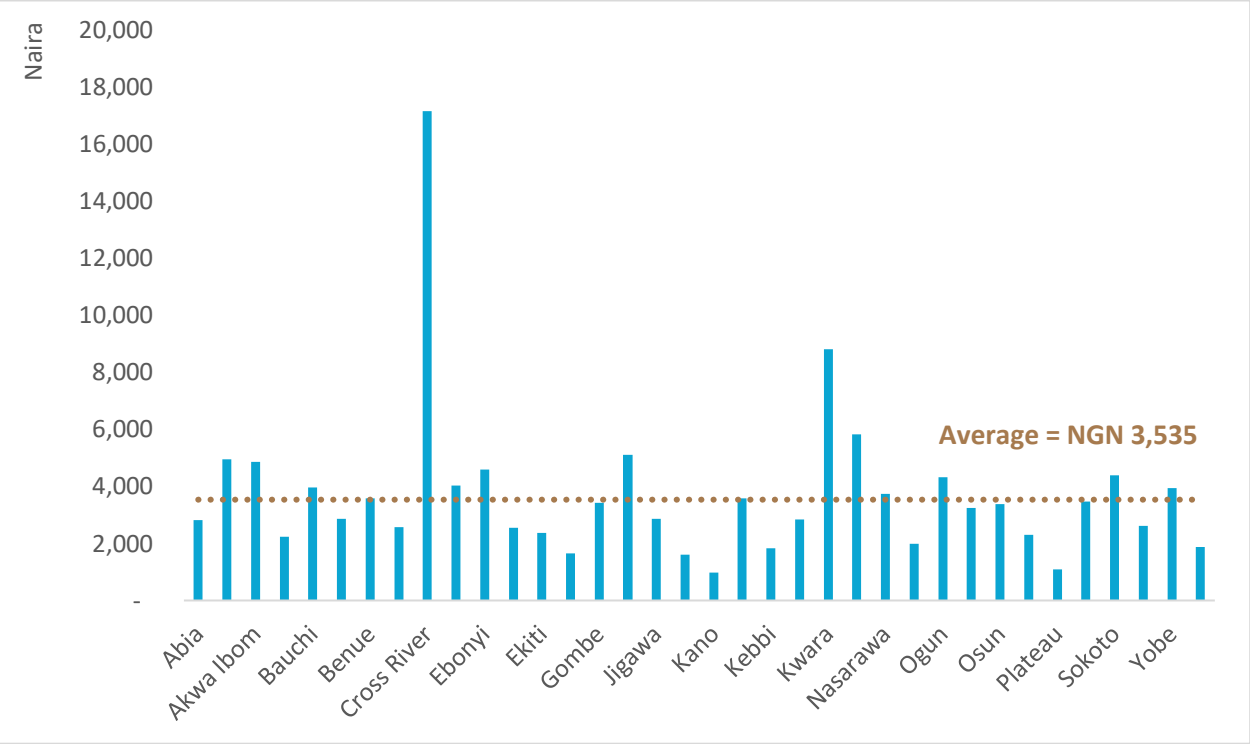
Health funds are highly fragmented across and within levels of government. Federal and state health budgets distribute resources across numerous budget-holding health agencies and hospitals (National Assembly 2020; Lagos State House of Assembly 2020). Additionally, government-run contributory health insurance schemes, including the NHIS and numerous SSHIS, manage separate pools for their small member populations. Finally, the BHCPF creates even more pools managed by state PHC Boards and SSHIS, with demand-side coverage and enrollment practices to be determined (Hafez 2018; Expert Interviews 2020; NPHCDA, NHIS, and NEMTC 2020).

Sub-national health funding pools are managed by state ministries of health, PHC Boards, health insurance schemes, and local health departments. SMOHs and PHC Boards receive allocations in state budgets that combine federally transferred revenue and, at times, state IGR. Where operational, SSHIS manage funds sourced from member contributions and earmarked general state revenue; they are also meant to manage separate BHCPF pools in the future. Local health departments manage modest pools of funds based on allocations by local government councils. States control resource flows to the local level and there are no standard rules or practices for resource distribution across localities. Moreover, beyond the general revenue sharing rules, there are no formalized mechanisms for risk- or needs-based reallocation across states or local government areas (Fatile and Ejalonibu 2015; Expert Interviews 2020). On average, states budgeted NGN 3,535 per capita for health in 2019.[14] These allocations varied significantly, ranging from less than NGN 1,000 in Kano to more than NGN 17,000 in Cross River (**Figure 10**).[15] These disparities cannot be explained by differences in health needs, providing further evidence that the rules and processes governing Nigeria's devolved system do not result in an equitable distribution of resources.

[14] Equivalent to roughly US$10 per capita.

[15] The budgets in many states, including Cross River, are based on unrealistic revenue projections (Uche et al. 2019). Therefore, conclusions about equity should be drawn cautiously from any state-to-state comparisons. Ideally, the distribution of actual health spending per capita across states would be analyzed over time to identify and track equity in health financing. Unfortunately, a source for health spending by state was not readily available and compiling the information state-by-state was not feasible for this study.

Figure 10. Per capita budget allocations to health by state, 2019



Sources: Uche et al. 2019 (for budget totals); National Bureau of Statistics 2017 (for populations)
Note: State-level populations are projections for 2016 based on the 2006 census.

PURCHASING

Purchasing captures how pooled funds are allocated to providers and used to pay for services. There are numerous health purchasers across Nigeria's three government tiers.

Purchasing Roles and Responsibilities

Federal and state purchasers are mutually autonomous, though their roles in purchasing PHC services are not always clearly differentiated. Purchasing from federal and state facilities is typically managed by agencies of the same tier (e.g., FMOH from federal hospitals and SMOHs from state facilities). Agencies in all three government tiers purchase various inputs for PHC services, including select commodities (federal), personnel (state), and operating expenses (local, if at all). In practice, local health authorities have minimal funding, and many PHC facilities only control the revenue they generate from user fees. Concurrent to budget flows, the NHIS and SSHIS contract with both public and private facilities, with the NHIS often purchasing from health maintenance organizations (HMOs) (Fatile and Ejalonibu 2015; NPHCDA 2015; Kress, Su, and Wang 2016; Odutolu et al. 2016; Eboreime et al. 2017; Bawas 2019; Expert Interviews 2020). Finally, the National Health Act of 2014 prescribes that 20% of BHCPF funds must be used for vaccines, essential drugs, and medical consumables;[16] 15% for the provision and maintenance of health facilities, equipment, and transportation; and 10% for human resource development.[17] As a statutory allocation, the

[16] If the federal government lacks sufficient funding for procuring vaccines for routine immunization, 10% can be reclaimed centrally to cover the shortfall.

[17] In total, these spending areas are meant to account for the 45% of BHCPF funds flowing through the NPHCDA (supply-side) gateway.

BHCPF may also carry over unused funds from one year to the next (NPHCDA 2015; Expert Interviews 2020).

Provider Payment and Contracting

State and local facilities receive resources in numerous forms, relying on in-kind transfers and user fees to cover non-personnel inputs. Health workers are salaried civil servants paid directly by state, and sometimes local, agencies, so there are minimal financial incentives for regular attendance, productivity, or generating good health outcomes. The federal government and donors procure a range of commodities, often for priority programs. These include antiretroviral drugs, HIV test kits, insecticide-treated bed nets, pulmonary diagnostic equipment and laboratory reagents, vaccines, cold chain equipment, and contraceptives. Federally procured commodities are distributed to states, which in turn deploy them to hospitals and PHC facilities. The federal government also directly funds portions of the PHC workforce, such as by supporting the salaries of midwives and community health extension workers. At the same time, 60% of health posts, 74% of other primary health centers, and 90% of secondary facilities reported collecting user fees, including for drugs and other commodities, lab tests, and delivery services (FMOH 2017b).

User fees are many facilities' only source of cash, and their contribution to total non-salary PHC revenues varies by state (e.g., 37% in Ekiti and 86% in Niger in 2015), as does the share of fees facilities retain (Kress, Su, and Wang 2016; FMOH 2017b; Alkenbrack et al. 2018; World Bank 2018c; Expert Interviews 2020). State governments generally provide operating budgets to their hospitals, while only a minority of PHC facilities receive direct allocations for operating expenses. In 2016, only one-third of surveyed primary and secondary facilities received any cash funding for operating expenses (FMOH 2017b). Facilities also collect revenue through government- and donor-financed schemes. For example, NHIS-contracted public facilities receive capitation payments for PHC services and fee-for-service payments for secondary care, which are additional to the budget allocations, in-kind support, and user fees they receive (Kress, Su, and Wang 2016; Alkenbrack et al. 2018; Hafez 2018). Previously, facilitates participating in pilots for performance-based financing (PBF) and decentralized facility financing (DFF) received quarterly payments from their respective state PHC Boards (World Bank 2012).

The emerging SSHIS may enable innovation in purchasing at sub-national levels. There is only a sub-national purchaser-provider split in states with an operational SSHIS. These schemes are meant to contract with both public and private providers, cover an evidenced-based service package, and employ a range of payment methods. For example, the SSHIS in several states—including Kaduna, Kano, and Oyo—plan to make capitation payments[18] for PHC services while centralizing procurement for key commodities (Ezekwem, Agbola, and Bodunrin 2020). Additionally, when states use money from the BHCPF, the SSHIS are meant to pay via capitation for PHC via and via fee-for-service for secondary care (NPHCDA, NHIS, and NEMTC 2020; Expert Interviews 2020). Whether these payment approaches yield desired improvements in service access, quality, and efficiency will depend on their detailed design and implementation.

Private health service providers deliver few publicly financed services. The NHIS contracts with both public and private service providers (Hafez 2018), as do some SSHIS. Per the National Health Act of 2014, the FMOH, SMOHs, local governments, and any public facility are permitted to enter agreements with private practitioners, facilities, or NGOs (Federal Republic of Nigeria 2014). However, it is unclear if there is adequate political appetite for more robust private sector engagement in health, and the government lacks a clear strategy in this area. Some take a cautionary view of the NHIS's experience with HMOs. Due to inadequate monitoring systems, it is difficult for the NHIS to track purchased services or measure actual delivery costs—both of which are critical challenges when facing providers that consistently complain

[18] Capitation refers to the prospective payment, usually monthly or quarterly, based on the number of members that have selected or been assigned to a provider.

about insufficient payment rates (Expert Interviews 2020). Nonetheless, the BHCPF operational guidelines state that "both public and private primary healthcare facilities shall be used as service delivery points [for the BMPHS," suggesting that the SSHIS will use BHCPF funds to purchase services from both sectors (NPHCDA, NHIS, and NEMTC 2020, 26). To what extent private providers will participate remains to be seen.

Benefits Package Design

Benefits packages vary across schemes and levels of the system. The National Health Act of 2014 establishes health entitlements for all citizens in the form of a Basic Minimum Package of Health Services (BMPHS), access to which is meant to be financed by the BHCPF (Federal Republic of Nigeria 2014). The NHIS, NPHCDA, and other health agencies designed the BMPHS under the auspices of the minister of health. The package includes preventive, promotive, curative, and rehabilitative services to be delivered at the primary and secondary levels (NPHCDA, NHIS, and NEMTC 2020). This package differs from the NPHCDA-defined Ward Minimum Package of Health Services, which indicates which services ought to be available across the three categories of PHC facilities, which include health posts, primary health clinics, and primary health care centers (Expert Interviews 2020; NPHCDA 2007). Additionally, the NHIS has its own comprehensive benefits package that covers out- and in-patient services across all levels of care, including costly services such as cancer treatment and renal dialysis (Hafez 2018). The SSHIS-managed contributory schemes also have benefits packages. Finally, work is ongoing to define, cost, and implement tailored versions of the BMPHS at the state level. Many states are likely to mimic the BMPHS, while some with sufficient resources may expand on it (Expert Interviews 2020; Joint Learning Network 2020).

Budget Execution

Fragmented pooling translates into fragmented fund flows. Refer back to **Figure 2**, which depicts financial and in-kind flows of government, private, and donor resources throughout the health system. At the federal level, funds flow from the FMOF to the FMOH and onward to federal facilities. Some federal health funds also flow to the NPHCDA, which then purchases inputs (e.g., vaccines) and gives them to states, which in turn distribute them for local use in PHC services. Similarly, state ministries of finance disburse funds to SMOHs and onward to state facilities. State funds also flow to PHC Boards that, like the NPHCDA, purchase inputs for in-kind distribution to primary health centers. Due to unrealistic budgets, states often resort to cash rationing, with line ministries jockeying via bureaucratic memos for fund releases. The NHIS and SSHIS, financed in part by the federal and state governments, respectively, issue payments per their contracts with providers, with NHIS funds often flowing through HMOs. Concurrently, states—in theory—authorize disbursement from the Joint Allocation Accounts to local health departments. The BHCPF channels federal funds to PHC facilities via state PHC Boards, and in the future, it will also provide funds that SSHIS will use to purchase the BMPHS from contracted providers. Finally, donors provide funds and in-kind contributions to federal and state health agencies and directly to facilities (Fatile and Ejalonibu 2015; Kress, Su, and Wang 2016; Alkenbrack et al. 2018; Hafez 2018; World Bank 2018c; Expert Interviews 2020).

Purchaser discretion varies by funding pool, shaped by public finance rules. Personnel spending is considered non-discretionary at all levels. Per the Fiscal Responsibility Act of 2007, the "sums appropriated for a specific purpose shall be used solely for the purpose specified in the Appropriation Act." The Federal Ministry of Budget and Planning can propose to the National Assembly, under "exceptional circumstances and in the overall public interest," certain reallocations (Federal Republic of Nigeria 2007, Part V, Section 27, Subsection 2). Consequently, federal and state ministries of health are bound by the input-based budget lines in legislated appropriations bills, with any major shifts requiring legislative approval. Ministries may not retain unspent funds, which are returned to the treasury at the end of each fiscal year (Hafez 2018). In contrast, parastatals such as the NHIS and SSHIS may deviate from standard PFM rules. They purchase according to their benefits and contracting policies, and they can retain funds from one year to the next (NPHCDA 2015; Expert Interviews 2020).

Local governments depend on state authorization to spend budgeted funds. Local government spending from Joint Allocation Accounts is governed at the state level, with practices varying by state (Fatile and Ejalonibu 2015; Kress, Su, and Wang 2016). For example, Ekiti state's Ministry of Local Government and Community Development controls local budgets and sets thresholds above which it must approve local expenditure.

Facilities have minimal autonomy in workforce management or use of budgeted funds. Federal and state health service commissions employ health workers in their respective facilities. Similarly, PHC workers are employed by state agencies, and decisions about employment and skills mix tend to be made by the state government rather than individual PHC facilities. It is difficult to determine what share of budgeted resources facilities control. For federally funded teaching hospitals, 98% of the 2020 budget was allocated to personnel (National Assembly 2020). In Lagos, most personnel costs are consolidated into a single budget line for the Health Service Commission, so they cannot be compared to the lump-sum allocations to individual hospitals indicated for recurrent and overhead costs. Moreover, locally managed PHC facilities are not visible in the state budget, which merely includes provisions for the Local Government Service Commission and Local Government Establishments (Lagos State House of Assembly 2020). In general, PHC facilities are not in states' charts of accounts. The BHCPF is meant to place more funds under direct facility control. For example, the portion of supply-side funds intended for vaccines, essential drugs, and medical consumables will be distributed as DFF, with the rest held centrally pending approval of investment plans. DFF transfers from the BHCPF will be the first time many PHC facilities receive any allocation or other direct government transfer (Expert Interviews 2020; NPHCDA, NHIS, and NEMTC 2020).

In contrast, facilities exert much more control over user fee revenue. In many states, PHC facilities are meant to remit user fees to their local government. However, compliance varies and the revenue is not systematically tracked. Consequently, facilities generally retain user fees for savings and use these fees according to their own priorities. In 2016, facility-generated revenue accounted for 3% of PHC resources in Ekiti and 12% in Niger. Other resources came from budget allocations for personnel (85% in Eikiti and79% in Niger), which are paid by state agencies, as well as from donors' cash and in-kind support to government (12% in Ekiti and 9% in Niger) (World Bank 2018c). User fee policies differ across states and they are rarely regulated or enforced, leaving facilities to set and update their own fee schedules. In some states, PHC facilities are expected to remit user fees to their local government for redistribution, but in practice, the revenue is not tracked, leaving state and local health officials with no visibility into facility revenue collection or use. Consequently, facilities have complete flexibility in how they use generated revenue. Typically, funds are used for a range of purposes, including to supplement health worker income, complete minor repairs and upgrades, and purchase materials and supplies. Facilities store their revenue on hand or in bank accounts for savings, or until disbursement (Kress, Su, and Wang 2016; FMOH 2017b; Expert Interviews 2020).

PHC facilities increasingly have their own bank accounts, but this is not consistent across the board. Some lack bank accounts and transact in cash, while others have several accounts; for example, one for NHIS payments, a second for drug recovery fund transactions, and a third for other revenue sources (FMOH 2017b; World Bank 2018c). Primary and secondary facilities participating in the PBF or DFF pilots received funds electronically into accounts managed by the facility officer in charge and the chair of the Ward Development Committee (World Bank 2018a). If facilities lack a bank account but want to receive BHCPF funds, they will need to "open and maintain bank accounts with commercial banks that are part of the Nigeria bankers' clearing house system" (NPHCDA, NHIS, and NEMTC 2020, 48). Community groups are meant to play an oversight role for facility accounts (Cotlear, Alawode, and Muchiri 2020).

REPORTING, OVERSIGHT, AND ACCOUNTABILITY

Reporting, oversight, and accountability mechanisms can enable governments, civil society, and citizens to monitor and evaluate performance of the health financing system at all levels.

There is strong separation between federal and state oversight. There are limits to what the federal government can require of state and local governments. At times, federal MDAs issue guidance for how sub-national levels should spend and account for funds, but states are free to choose whether to adopt them or not. More assertive federal action on this matter is politically and constitutionally sensitive (Expert Interviews 2020).

Financial reporting and monitoring systems are stronger at the federal than the state and local levels. The federal government began using GIFMIS in 2012, and by 2018, the system was mainstreamed across most federal MDAs for budget preparation and accounting. It captured payments representing 93% of federal expenditure but had not yet been used to monitor budget execution (World Bank 2018b). State resource tracking systems are generally more rudimentary, and state governments tend to be more attentive to and accountable for use of donor funds, whereas there is less transparency at all levels for domestic financing (Expert Interviews 2020). Other than for payroll, local budgeting and accounting are generally conducted manually and plagued by inaccuracies and errors. In-kind inputs are only recorded when received from the federal government, not donors, and there is no regular financial reporting mechanism from PHC facilities to local governments (World Bank 2018c). Health facilities are supposed to maintain records or registers of expenditure, including transaction dates, reasons for expenditures, amounts disbursed, and balance on hand. In 2016, of the surveyed primary and secondary facilities, only 20% produced a register, only 15% had up-to-date records, and only in 8% did the records reconcile with cash on hand (FMOH 2017b). Likewise, data on user fee revenue are not systematically collected and may not appear in budget or financial reports. Whether facilities maintain and report manual transaction records varies by state (World Bank 2018c).

Information about health budget performance is fragmented and incomplete. There is no consolidated picture of budget execution in the health sector across all levels of government. At the federal level, only capital expenditure is reported in the annual budget implementation report. In 2018, 61% of the appropriated funds for capital expenditure in health were spent, corresponding to 83% of funds that were released. The report does not disaggregate between domestically raised and external funds (Budget Office of the Federation 2019). A new policy and its associated guidelines for financial transparency were issued at the end of 2019 (Federal Government of Nigeria 2019); yet, their implementation via a new online portal could improve the availability of timely federal budget performance information.[19] Some states' annual audited financial statements include budget performance information,[20] including by sector or MDA (or both) and disaggregated by spending category (personnel, other recurrent, and capital). For example, in 2018, the state auditor general found that Lagos spent 78% of its health budget, including overspending on personnel (110%) and underspending on capital projects (44%). Its report did not analyze the drivers of

[19] In accordance with the new policy, it appears that the federal government aims to publish daily, monthly, quarterly, and annual expenditure data to the Open Treasury Portal (**https://opentreasury.gov.ng**). Currently the site contains partial documentation from 2018 onward. For example, monthly reports on FMOH budget performance are available from 2019, but not quarterly or annual.

[20] A systematic analysis of state financial reports was not conducted for this case study. A cursory review of documents from Lagos and Osun (Lagos State Auditor General 2019; Osun State Accountant-General 2020)(Lagos State Auditor General 2019; Osun State Accountant-General 2020) confirmed that budget performance information is available online for those states, though 2019 information could not be found for Lagos, and. Additionally, across the states data are not presented in a standard format, suggesting that compiling all details for health both within and across states would require considerable effort.

budget performance, though aggregate data on revenue performance suggests that approved budgets may have been based on unrealistic IGR forecasts (Lagos State Auditor General 2019).

There is no systematic analysis of health budget performance across levels of government or states. Federal quarterly and year-end budget implementation reports focus on the aggregate federal budget, with only minor analysis of MDA-specific capital budget utilization (Budget Office of the Federation 2019). Monthly GIFMIS reports, now available on the Open Treasury Portal, provide considerably more detail, including for the FMOH overall and each of its subsidiary budget codes. For example, in 2019 the FMOH spent 79% of its budget overall, with individual entities—such as the National Blood Transfusion Service and Dental Therapists Registration Board— spending anywhere from 26% to 117%, respectively. Separate reports break budget performance down by economic classification. For instance, in 2019 only 53% of funds budgeted for the purchase of medical equipment were spent. Yet despite increasing data availability, the FMOH does not conduct an annual review of health sector budget performance and there has not been an annual health report since 2008 (Hafez 2018). Although some states report health budget performance in their annual financial statements, there are no formalized mechanisms or organizations charged with consolidating health spending information across levels of government, geographies, or types of services (Hafez 2018; World Bank 2018c). Civil society may be taking a more active interest in scrutinizing health budgets and performance. In 2019, the Lagos-based BudgIT, a social advocacy organization, used expenditure data for the first time in its annual report on state fiscal sustainability and examined prioritization of health in state budgets (Uche et al. 2019).

Federal and state governments publish annual budgets and audit reports, though with gaps and delays. The Budget Office of the Federation publishes documents online pertaining to federal budget preparation,[21] including the budget call circular, budget proposals, and the approved annual budget. MDA-specific budget documents are available for some, but not all, years. States also publish budget documents, including on a general government website for Osun[22] and one maintained by the Lagos budget ministry.[23] The 1999 constitution requires all public accounts of the federation to be audited and reported to the Auditor-General, and the Fiscal Responsibility Act of 2007 specifies that audited federal accounts must be published within six months of the end of the financial year (Constitution of the Federal Republic of Nigeria 1999; Federal Republic of Nigeria 2007). However, the most recent federal financial statements and audit report available online are from 2017. Audited financial statements from both Lagos and Osun for 2019 are available on their respective websites.

There are several health sector accountability structures whose functionality and effectiveness vary widely. These structures include:

- Facility Health Committees and Ward Development Committees, both meant to improve community participation in health system design, accountability, and resource mobilization;
- State-led Accountability Mechanisms (SLAMs), which oversee the accountability structures in the state and participate in resource tracking and mobilization. SLAMs comprise government officials, civil society organizations, hospital management boards, professional bodies, and the media; and
- At the federal level, the Health Sector Reform Coalition, a partnership of public, private, and non-governmental organizations, recently developed a new health financing accountability framework that includes indicators, targets, data sources, and stakeholder responsibilities.

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[21] See **https://www.budgetoffice.gov.ng/index.php/resources/internal-resources/budget-documents**.

[22] The State Government of Osun posts budget documents at **https://osun.gov.ng/all-budget-documents/**.

[23] The Lagos Ministry of Economic Planning and Budget posts them at **https://mepb.lagosstate.gov.ng/budget-documents/y2008-2020-budget**.

Anecdotally, there is evidence that some Facility Health Committees help to improve health worker absenteeism and improve drug availability. Some SLAMs may have also helped to secure greater state budget allocations to health (Expert Interviews 2020).

DISCUSSION

Federalism shapes every aspect of health financing in Nigeria. The amount of public funds available for health, and how they are raised, stem from the revenue authorities granted to each tier of government, the rules governing how federally collected revenue is shared with states, the power dynamics that often prevent revenue sharing with local governments, and priorities set at all levels. Low levels of government health expenditure drive reliance on OOP spending by households, leaving most financially vulnerable to health shocks. Modest government funds are fragmented across numerous pools managed at the federal and state levels, doing little to improve financial protection or the fairness of resource allocation. These are among the many reasons why, despite having reached middle-income status, Nigeria has outperformed very few countries in progress toward UHC (World Bank 2020). Although no single study can do justice to the complexity and diversity of health financing practices in Nigeria, this review suggests several notable themes in relation to the interplay among devolution, health financing, and PFM:

1. **The revenue sharing rules and budgeting practices within Nigeria's federal system hinder sufficient and equitable government financing for health.** The revenue sharing rules reinforce, rather than correct, for existing inequalities in economic activity, natural resource endowment, and fiscal capacity. Consequently, most state and local governments lack sufficient resources to fulfill the wide-ranging functions with which they are tasked, including in health. This scarcity of resources at sub-national levels, combined with low prioritization of health in government spending, leads to significant reliance on user fees to finance health facility operations. In fact, even though the government's nominal per capita health spending increased between 2006 and 2016, the federal tier's increase in real per capita expenditure were more or less offset by decreases at the state and local levels. Moreover, the share of health expenditures from OOP spending increased during the same period (FMOH 2010; 2017a). As a result, access to services remains rationed primarily based on ability to pay rather than need, in turn contributing to the high incidence of catastrophic health expenditure (Okedo-Alex et al. 2019) and inequitable access to essential services (Ushie, Udoh, and Ajayi 2019). Full implementation of the BHCPF may improve this situation.

2. **Health reforms are slow to overcome the severe fragmentation throughout the health budget cycle.** States jealously guard their fiscal autonomy and their planning, management, and implementation of government spending is largely disconnected from corresponding processes at the federal level. There is minimal coordination or information sharing across levels of government, and the federal tier cannot legally impose many mechanisms or standards for reporting, oversight, or accountability on the sub-national levels. Implementation has been slow of both the Primary Health Care Under One Roof policy and the 2014 National Health Act. The former entrenches and streamlines state-level governance of PHC services—further limiting the local tier's role—while the latter seeks to improve access to basic services through the federally financed BHCPF. It is not yet clear whether these initiatives, combined with the ongoing implementation of state-level contributory health insurance schemes, are reducing or exacerbating the disparate flows of funds and information throughout the health system. In the meantime, state-level per capita allocations to health remain highly inequitable, and there is

great need for a more detailed analysis of both health budgets and expenditure at state and local levels.

3. **Improvements to PFM are slow to diffuse from the federal to sub-national levels.** The federal government continues to implement an ambitious array of reforms to budget preparation and financial accountability, highlighted by the adoption of GIFMIS and rollout of zero-based budgeting across the federal tier. The GIFMIS has increased transparency and efficiency by streamlining and digitizing routine requests and approvals for releasing funds to implementing ministries. In contrast, state and local governments rely on less sophisticated systems and only some frontline health facilities even try to record user fee revenue and the spending it enables. Furthermore, the lack of standardized and universally automated budget and expenditure data hinder any efforts to consolidate budget information across geographies and government tiers; to harmonize planning and better align health budgets with shared policy objectives; and to monitor health spending and hold health officials accountable at the state and local levels.

4. **The prevailing rules and practices in Nigeria's three-tiered system of government have not (yet) placed substantial resources under the control of local facilities and officials.** Personnel and supplies flow mostly in-kind to PHC facilities, which rely primarily on user fees for any discretionary spending. Additionally, most PHC facilities generally cannot alter their workforce capacity and skillset in response to local health needs. Local governments manage less than one-tenth of government health expenditures despite their direct responsibility for PHC delivery. Looking forward, the still-nascent BHCPF and SSHIS have the potential to channel significantly more funds directly to frontline facilities, due in part to the fact that many already have bank accounts. Encouragingly, there is mounting evidence that direct facility financing can significantly improve health system performance (Khanna et al. 2021). However, the BHCPF and many state schemes are not yet fully operational. Therefore, it is unclear whether new financing arrangements will provide enough resources or create the right incentives to achieve more equitable access to high-quality PHC services.

5. **Finally, the health financing system is evolving to work more around than through the federal structure.** In theory, revenue sharing rules could provide state and local governments enough resources to fulfill their many functions, and in turn they would allocate sufficient funding to health to ensure that all Nigerians have affordable access to high-quality services, especially PHC. Such a reality is unlikely to emerge given numerous political and practical constraints. For instance, the federal and state tiers jealously guard revenue, and most state and local governments lack capacity and systems for needs-based planning and budgeting. Instead, the country's strategy to increase PHC funding relies mainly on the BHCPF, an earmarked allocation of federal revenue designed to bypass the intergovernmental revenue sharing system and separately channel money via state agencies to PHC facilities.

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ANNEX 1: LIST OF INTERVIEWED INDIVIDUALS

This annex contains a list of individuals interviewed by the author between August 5 and October 21, 2020. For each interview, a semi-structured questionnaire was prepared based on the subject's expertise and information gaps following the desk review.

Name	Affiliation	Interview date
Alawode, Gafar (Dr.)	Managing Partner, DGI Consult, Nigeria	October 21,2020
Angale, Ibrahim (Dr.)	Health Financing Advisor, Palladium, Nigeria	October 12, 2020
Baruwa, Elaine (Dr.)	Principal Associate/Health Economist, Abt Associates, USA	October 16, 2020
Ezekwe, Ezinne (Ms.)	Senior Program Associate, Results for Development, USA	October 5, 2020
Obi, Felix (Mr.)	Senior Program Officer, Results for Development, Nigeria	October 5, 2020
Tegbe, Muyiwa (Dr.)	Senior Technical Advisor, ThinkWell, USA	August 5, 2020

ANNEX 2: FEDERAL BUDGET EXTRACT

This annex contains the first page of the FMOH's overall budget, summarizing total allocations to the ministry and subsidiary budget units, and the first two pages of the budget for the ministry's own activities. These pages were extracted from National Assembly (2019).

Federal Government of Nigeria

SUMMARY BY MDAs

2019 APPROPRIATION ACT

NO	CODE	MDA	TOTAL PERSONNEL	TOTAL OVERHEAD	TOTAL RECURRENT	TOTAL CAPITAL	TOTAL ALLOCATION
1.	0521	FEDERAL MINISTRY OF HEALTH	311,245,805,391	4,371,538,665	315,617,344,056	57,085,655,234	372,702,999,290
			311,245,805,391	**4,371,538,665**	**315,617,344,056**	**57,085,655,234**	**372,702,999,290**

SUMMARY BY FUNDS

2019 APPROPRIATION ACT

NO	CODE	FUND	TOTAL ALLOCATION
1.	021	MAIN ENVELOP - PERSONNEL	311,245,805,391
2.	022	MAIN ENVELOP - OVERHEAD	4,371,538,665
3.	031	CAPITAL DEVELOPMENT FUND MAIN	57,085,655,234
			372,702,999,290

FEDERAL MINISTRY OF HEALTH

2019 APPROPRIATION ACT

NO	CODE	MDA	TOTAL PERSONNEL	TOTAL OVERHEAD	TOTAL RECURRENT	TOTAL CAPITAL	TOTAL ALLOCATION
1.	0521001001	FEDERAL MINISTRY OF HEALTH - HQTRS	7,411,093,350	476,513,638	7,887,606,988	10,553,270,943	18,440,877,931
2.	0521001002	NATIONAL BLOOD TRANSFUSION SERVICE (NBTS)	529,951,945	150,000,000	679,951,945	400,000,000	1,079,951,945
3.	0521002001	NATIONAL HEALTH INSURANCE SCHEME	0	0	0	648,953,434	648,953,434
4.	0521003001	NATIONAL PRIMARY HEALTH CARE DEVELOPMENT AGENCY	2,221,880,412	209,330,979	2,431,211,391	16,006,999,994	18,438,211,385
5.	0521005001	NATIONAL ARBOVIRUS AND VECTOR RESEARCH	106,738,161	1,647,534	108,385,695	50,000,000	158,385,695
6.	0521006001	RADIOGRAPHERS REGISTRATION BOARD	278,336,503	3,950,354	282,286,857	20,000,000	302,286,857
7.	0521007001	DENTAL TECHNOLOGY REGISTRATION BOARD	170,296,009	1,430,412	171,726,421	70,000,000	241,726,421
8.	0521008001	HEALTH RECORDS REGISTRATION BOARD	297,152,815	1,541,580	298,694,395	20,000,000	318,694,395
9.	0521009001	OPTOMETRIST AND DISPENSING OPTICIANS BOARD OF NIGERIA	231,577,119	1,559,546	233,136,665	65,000,000	298,136,665
10.	0521010001	COMMUNITY HEALTH PRACTITONERS REGISTRATION BOARD	79,887,588	1,765,931	81,653,519	10,000,000	91,653,519
11.	0521011001	NURSING AND MIDWIFERY COUNCIL	299,084,323	3,132,062	302,216,385	533,000,000	835,216,385
12.	0521012001	PHARMACIST COUNCIL OF NIGERIA COUNCIL	758,471,294	8,551,000	767,022,294	50,000,000	817,022,294
13.	0521013001	MEDICAL AND DENTAL COUNCIL OF NIGERIA	282,400,932	10,325,369	292,726,301	10,000,000	302,726,301
14.	0521014001	NATIONAL AGENCY FOR FOOD AND DRUG ADMINISTRATION AND CONTROL	6,114,793,422	7,568,033	6,122,361,455	40,364,058	6,162,725,513
15.	0521015001	MEDICAL REHABILITATION THERAPY BOARD	312,593,960	30,874,671	343,468,631	118,000,000	461,468,631
16.	0521016001	FEDERAL COLLEGE OF DENTAL TECHNOLOGY AND THERAPY, ENUGU	541,112,072	44,000,000	585,112,072	165,000,000	750,112,072
17.	0521017001	ENVIRONMENTAL HEALTH OFFICERS TUTORS-IBADAN	34,893,487	8,888,262	43,781,749	5,000,000	48,781,749
18.	0521018001	NURSE TUTOR TRAINING - ENUGU	18,616,218	1,651,498	20,267,716	5,000,000	25,267,716
19.	0521019001	NURSE TUTOR PROGRAMME AKOKA LAGOS	105,082,803	2,105,000	107,187,803	5,000,000	112,187,803
20.	0521020001	NURSE TUTOR TRAINNING KADUNA	62,214,638	2,034,656	64,249,294	10,000,000	74,249,294
21.	0521021001	NURSE TUTOR TRAINNING IBADAN	26,659,352	1,651,494	28,310,846	7,098,000	35,408,846
22.	0521022001	NATIONAL POST GRADUATE MEDICAL COLLEGE OF NIGERIA-IJANIKIN LAGOS	255,310,447	16,626,390	271,936,837	200,000,000	471,936,837
23.	0521024001	PHC TUTORS PROGRAMME,'UCH-IBADAN	27,226,184	1,535,053	28,761,237	9,528,750	38,289,987
24.	0521024002	PHC TUTORS PROGRAMME,KADUNA POLYTECHNIC	0	2,577,362	2,577,362	10,500,000	13,077,362
25.	0521025001	COMMUNITY HEALTH TUTOR PROGRAMME UCH	31,356,086	1,695,207	33,051,293	9,922,500	42,973,793
26.	0521026001	UNIVERSITY COLLEGE HOSPITAL IBADAN	14,582,330,951	104,701,272	14,687,032,223	830,000,000	15,517,032,223
27.	0521026002	LAGOS UNIVERSITY TEACHING HOSPITAL	7,155,460,640	100,301,959	7,255,762,599	270,000,000	7,525,762,599
28.	0521026003	AHMADU BELLO UNIVERSITY TEACHING HOSPITAL	7,312,045,790	77,711,932	7,389,757,722	730,000,000	8,119,757,722
29.	0521026004	UNIVERSITY OF NIGERIA TEACHING HOSPITAL, ENUGU	12,520,698,700	67,203,564	12,587,902,264	260,000,000	12,847,902,264
30.	0521026005	UNIVERSITY OF BENIN TEACHING HOSPITAL	8,812,195,218	54,204,869	8,866,400,087	460,000,000	9,326,400,087
31.	0521026006	OBAFEMI AWOLOWO UNIVERSITY TEACHING HOSPITAL	8,265,743,473	89,216,093	8,354,959,566	470,000,000	8,824,959,566
32.	0521026007	UNIVERSITY OF ILORIN TEACHING HOSPITAL, ILORIN	8,542,008,146	54,885,939	8,596,894,085	260,000,000	8,856,894,085
33.	0521026008	JOS UNIVERSITY TEACHING HOSPITAL	7,884,350,277	66,253,910	7,950,604,187	240,000,000	8,190,604,187
34.	0521026009	UNIVERSITY OF PORT-HARCOURT TEACHING HOSPITAL	8,687,051,205	64,505,000	8,751,556,205	445,000,000	9,196,556,205
35.	0521026010	UNIVERSITY OF CALABAR TEACHING HOSPITAL	9,769,664,158	98,863,376	9,868,527,534	229,000,000	10,097,527,534

0521001001	FEDERAL MINISTRY OF HEALTH		2019 APPROPRIATION ACT
CODE	LINE ITEM		AMOUNT
2	EXPENDITURE		18,440,877,931
21	PERSONNEL COST		7,411,093,350
2101	SALARY		6,168,865,302
210101	SALARIES AND WAGES		6,168,865,302
21010101	SALARY		6,168,865,302
2102	ALLOWANCES AND SOCIAL CONTRIBUTION		1,242,228,048
210201	ALLOWANCES		474,032,835
21020101	NON REGULAR ALLOWANCES		474,032,835
210202	SOCIAL CONTRIBUTIONS		768,195,213
21020201	NHIS		307,278,085
21020202	CONTRIBUTORY PENSION - EMPLOYER'S CONTRIBUTION		460,917,128
22	OTHER RECURRENT COSTS		476,513,638
2202	OVERHEAD COST		476,513,638
220201	TRAVEL& TRANSPORT - GENERAL		81,856,000
22020101	LOCAL TRAVEL & TRANSPORT: TRAINING		22,596,000
22020102	LOCAL TRAVEL & TRANSPORT: OTHERS		26,380,000
22020103	INTERNATIONAL TRAVEL & TRANSPORT: TRAINING		6,500,000
22020104	INTERNATIONAL TRAVEL & TRANSPORT: OTHERS		26,380,000
220202	UTILITIES - GENERAL		34,600,000
22020201	ELECTRICITY CHARGES		7,600,000
22020203	INTERNET ACCESS CHARGES		7,500,000
22020204	SATELLITE BROADCASTING ACCESS CHARGES		6,000,000
22020205	WATER RATES		5,000,000
22020206	SEWERAGE CHARGES		8,500,000
220203	MATERIALS & SUPPLIES - GENERAL		42,300,000
22020301	OFFICE STATIONERIES / COMPUTER CONSUMABLES		7,000,000
22020302	BOOKS		6,000,000
22020303	NEWSPAPERS		4,800,000
22020304	MAGAZINES & PERIODICALS		6,500,000
22020305	PRINTING OF NON SECURITY DOCUMENTS		4,000,000
22020306	PRINTING OF SECURITY DOCUMENTS		6,000,000
22020307	DRUGS & MEDICAL SUPPLIES		8,000,000
220204	MAINTENANCE SERVICES - GENERAL		57,500,000
22020401	MAINTENANCE OF MOTOR VEHICLE / TRANSPORT EQUIPMENT		8,000,000
22020402	MAINTENANCE OF OFFICE FURNITURE		8,000,000
22020403	MAINTENANCE OF OFFICE BUILDING / RESIDENTIAL QTRS		7,000,000
22020404	MAINTENANCE OF OFFICE / IT EQUIPMENTS		7,500,000
22020405	MAINTENANCE OF PLANTS/GENERATORS		18,000,000
22020406	OTHER MAINTENANCE SERVICES		9,000,000
220205	TRAINING - GENERAL		26,000,000
22020501	LOCAL TRAINING		18,000,000
22020502	INTERNATIONAL TRAINING		8,000,000
220206	OTHER SERVICES - GENERAL		38,000,000
22020601	SECURITY CHARGES		16,000,000
22020603	OFFICE RENT		12,000,000
22020606	CLEANING AND FUMIGATION SERVICES		10,000,000
220207	CONSULTING & PROFESSIONAL SERVICES - GENERAL		38,750,000
22020701	FINANCIAL CONSULTING		4,000,000
22020702	INFORMATION TECHNOLOGY CONSULTING		6,000,000
22020703	LEGAL SERVICES		3,000,000
22020708	MEDICAL CONSULTING		25,750,000
220208	FUEL & LUBRICANTS - GENERAL		20,000,000
22020801	MOTOR VEHICLE FUEL COST		12,000,000
22020803	PLANT / GENERATOR FUEL COST		8,000,000
220209	FINANCIAL CHARGES - GENERAL		14,000,000
22020901	BANK CHARGES (OTHER THAN INTEREST)		6,000,000
22020902	INSURANCE PREMIUM		8,000,000
220210	MISCELLANEOUS		123,507,638
22021003	PUBLICITY & ADVERTISEMENTS		3,000,000
22021006	POSTAGES & COURIER SERVICES		3,000,000
22021007	WELFARE PACKAGES		40,000,000
22021009	SPORTING ACTIVITIES		39,507,638
22021014	ANNUAL BUDGET EXPENSES AND ADMINISTRATION		12,000,000
22021029	MONITORING ACTIVITIES & FOLLOW UP		8,000,000
22021030	PROMOTION, RECRUITMENT & APPOINTMENT		18,000,000
23	CAPITAL EXPENDITURE		10,553,270,943
2301	FIXED ASSETS PURCHASED		1,807,678,490
230101	PURCHASE OF FIXED ASSETS - GENERAL		1,807,678,490
23010105	PURCHASE OF MOTOR VEHICLES		20,000,000
23010113	PURCHASE OF COMPUTERS		42,133,333
23010122	PURCHASE OF HEALTH / MEDICAL EQUIPMENT		1,745,545,157
2302	CONSTRUCTION / PROVISION		104,885,514
230201	CONSTRUCTION / PROVISION OF FIXED ASSETS - GENERAL		104,885,514
23020101	CONSTRUCTION / PROVISION OF OFFICE BUILDINGS		104,885,514
2303	REHABILITATION / REPAIRS		659,941,992

0521001001	FEDERAL MINISTRY OF HEALTH		2019 APPROPRIATION ACT
CODE	LINE ITEM		AMOUNT
230301	**REHABILITATION / REPAIRS OF FIXED ASSETS - GENERAL**		**659,941,992**
23030105	REHABILITATION / REPAIRS - HOSPITAL / HEALTH CENTRES		576,516,900
23030121	REHABILITATION / REPAIRS OF OFFICE BUILDINGS		83,425,092
2304	**PRESERVATION OF THE ENVIRONMENT**		**3,000,000**
230401	**PRESERVATION OF THE ENVIRONMENT - GENERAL**		**3,000,000**
23040104	INDUSTRIAL POLLUTION PREVENTION & CONTROL		3,000,000
2305	**OTHER CAPITAL PROJECTS**		**7,977,764,947**
230501	**ACQUISITION OF NON TANGIBLE ASSETS**		**7,977,764,947**
23050101	RESEARCH AND DEVELOPMENT		1,489,307,063
23050102	COMPUTER SOFTWARE ACQUISITION		17,735,200
23050103	MONITORING AND EVALUATION		113,790,000
23050171	GOVERNANCE/INSTITUTIONAL REFORM		6,356,932,684
	TOTAL PERSONNEL		**7,411,093,350**
	TOTAL OVERHEAD		**476,513,638**
	TOTAL RECURRENT		**7,887,606,988**
	TOTAL CAPITAL		**10,553,270,943**
	TOTAL ALLOCATION		**18,440,877,931**
0521001001	**FEDERAL MINISTRY OF HEALTH - HQTRS**		
CODE	PROJECT NAME	TYPE	AMOUNT
ERGP25112202	IMNCH STRATEGY - ARTICULATION OF POLICY AND STRATEGY PLANS OF NEWBORN IN CHILD HEALTH IN LINE WITH SDG TARGETS ON INTEGRATED MANAGEMENT OF CHILDHOOD ILLNESSES (IMCI) FOLLOW UP, INTEGRATED SUPPORTIVE SUPERVISION (ISS) AND KANGAROO MOTHER CARE (KMC)	ONGOING	425,057,243
ERGP25112207	IMPROVE FAMILY PLANNING SERVICES THROUGH CONTRACEPTIVES USE INTERVENTIONS AND COUNTERPART FUNDING	ONGOING	300,000,000
ERGP25112220	PROCUREMENT OF READY-TO-USE THERAPEUTIC FOOD TO BE DISTRIBUTED TO 6 GEOPOLITICAL ZONES INCLUDNG ESTABLISHMENT OF CMAM SITES	ONGOING	180,000,000
ERGP25112228	CONDUCT RAPID ASSESSMENT OF HEALTH PROMOTION PROGRAMMES IN 555 PHCS	ONGOING	6,271,239
ERGP25112236	BILATERAL/ MULTILATERAL/ WHO GENEVA/WHA GENEVA /UNGA NEW YORK/WAHO EQUITORIAL GUINEA; WHO REGIONAL MEETING IN COTE D'IVOIRE COOPERATION ACTIVITIES INCLUDING INTERNATIONAL STATUTORY OBLIGATIONS WHICH NIGERIA IS SIGNATORY AND MANDATORY TO PARTICIPATE	ONGOING	100,260,000
ERGP25112239	HEALTH DEVELOPMENT PARTNERS COORDINATION ACTIVITIES FOR BETTER EFFICIENCY AND ACHIEVING SDGS RELATED ACTIVITIES	ONGOING	8,300,000
ERGP25112243	IMPLEMENTATION OF NATIONAL HEALTH RESEARCH POLICY AND PRIORITIES ACCORDING TO NHACT 2014; ESTABLISHMENT OF ESSENTIAL NATIONAL HEALTH RESEARCH REGISTRY IN FMOH	ONGOING	16,128,400
ERGP25112251	COLLECTION, COLLATION AND ARCHIVING OF BOTH PAST AND ON-GOING RESEARCH OUTCOMES IN THE COUNTRY; ESTABLISHMENT OF ETHICAL COMMITTEE IN ALL THE STATES MINISTRIES OF HEALTH.	ONGOING	7,048,391
ERGP25112255	MONITORING AND EVALUATION OF CAPITAL PROJECTS IN THE HEALTH SECTOR	ONGOING	34,000,000
ERGP25112257	STRATEGIC PURCHASE OF A GUARATNTEED PACKAGE OF PREVENTIVE, PROMOTIVE AND CURATIVE CARE FOR NIGERIANS AND THE PROVISION OF OPERATING BUDGETS TO PHCSVIA ELECTRONIC TRANSFERS, UNDER THE BASIC HEALTH CARE PROVISION FUND (BHCPF)	ONGOING	20,000,000
ERGP25112261	STRENGTHENING OF THE NATIONAL HEALTH INFORMATION MANAGEMENT SYSTEM, STRENGTHENING AND IMPROVING SURVEY COORDINATION IN THE HEALTH SECTOR	ONGOING	25,000,000
ERGP25112268	FIRST STEP OF MULTI-YEAR CAPACITY STRENGTHENING FOR THE DEPARTMENT FOLLOWING A FUNCTIONAL REVIEW AND CAPACITY ASSESSMENT CARRIED OUT	ONGOING	20,000,000
ERGP25112271	CONDUCT OF HEALTH WORKFORCE PROFILE, CAPACITY BUILDING, REVITALIZE NATIONAL COLLABORATING TRAINING CENTRES, AND NATIONAL PUBLIC HEALTH TRAINING INITIATIVE	ONGOING	11,905,851
ERGP25112295	INSPECTORATE VISITS TO 54 TERTIARY HOSPITALS AND MONITORING OF COMPLIANCE WITH REGULATORY STANDARDS	ONGOING	34,000,000
ERGP25112298	ESTABLISHMENT OF THE NATIONAL BLOOD TRANSFUSION SERVICE COMMISSION (NBTS) SERVICES; PRINTING, INSTALLATIONS, CAPACITY BUILDING, RECRUITMENT OF VOLUNTEERS, WORLD BLOOD DONOR DAY COMEMORATIONR, STAKEHOLDERS MEETINGS, AND PRODUCTION OF POLICY AND OPERATIONAL GUIDELINES ETC	ONGOING	60,699,442
ERGP25112305	PROCUREMENT OF NATIONAL BLOOD TRANSFUSION SERVICE (NBTS) EQUIPMENTS, KITS, OFFICE EQUIPMENTS, AND CONSUMMABLES	ONGOING	20,000,000
ERGP25112303	NATIONAL CERVICAL CANCER SCREENING SCALE UP PROJECT, TV AND RADIO CANCER AWARENESS AND NUCLEAR MEDICINE MANAGEMENT COMMITTEE (CANCER CONTROL PROGRAMME)	ONGOING	70,000,000
ERGP25112315	TRAINING OF OXYGEN UTILIZATION AND ADVOCACY TO IMPLEMENT OXYGEN POLICY	ONGOING	4,000,000
ERGP25112324	CANCER REGISTRATION,PALLIATIVE HEALTH CARE POLICY , ONCOLOGY FOR NON-ONCOLOGY TRAINING AND NATIONAL CANCER PLAN AND CANCER PREVENTION CONTROL AND MANAGEMENT PROGRAMME (CANCER CONTROL PROGRAMME) INTERNATIONL=AL CANCER WEEK COMMEMORATION CANCER CONTROL PROGRAM	ONGOING	20,000,000
ERGP25112325	OPERATIONALISATION OF THE COMMITTEES ON NHACT; NATIONAL TERTIARY HEALTH INSTITUTIONS STANDARDS COMMITTEE AND TISSUE TRANSPLANTATION ETHICS COMMITTEE, NATIONAL RESEARCH COMMITTEE, SUPPORT FOR THE SECRETARIAT AND LOGISTICS	ONGOING	20,000,000
ERGP25112326	DEVELOPMENT OF THE THREE YEAR ACTION PLAN ON ERADICATION OF NOMA (FACIAL ULCER) IN NIGERIA, PRINTONG PF IEC MATERIALS FOR CAMPAIGNS OF NOMA PREVENTION IN FOUR GEOGRAPHICAL REGIONS, CAPACITY BUILDING OF STAFF OF THE NOMA HOSPITAL IN SOKOTO TO BETTER MANAGE NOMA CASES, COMMEMORATION OF WORLD NOMA DAY	ONGOING	10,000,000

ANNEX 3: LAGOS STATE BUDGET EXTRACT

This annex contains the Lagos state health budget as extracted from Lagos State House of Assembly (2020).

LAGOS STATE HOUSE OF ASSEMBLY
Y2020 APPROVED BUDGET
OMNIBUS

ANNEXURE II

			Y2020 REVENUE(CRF) N	Y2020 REVENUE DEDICATED (RECURRENT) N	Y2020 DEDICATED EXPENDITURE (RECURRENT) N	Y2020 PERSONNEL COST (MOET&P) N	Y2020 OVERHEAD COST N	Y2020 CAPITAL RECEIPTS (INFLOW) N	Y2020 CAPITAL DEVELOPMENT (OUTFLOW) N	Y2020 CAPITAL EXPENDITURE N	Y2020 SUBVENTION N
Housing and Community Amenities			**58,817,742,227**	**500,000,000**	**500,000,000**	**2,275,801,028**	**672,624,526**	**14,798,100,781.68**	**14,798,100,781.68**	**28,510,851,454**	**1,194,430,858**
Housing			**3,604,000,000**	-	-	**208,490,000**	**110,000,000**	**7,688,100,781.68**	**7,688,100,781.68**	**9,187,586,857**	**250,990,000**
280	014	Ministry of Housing	**872,098,218**			**208,490,000**	**110,000,000**	**5,868,800,000.84**	**7,688,100,781.68**	**9,082,473,864**	
281		Lagos State Real Estate Transaction Department (LASRETRAD)	**1,527,901,782**							**70,000,000**	**180,000,000**
		Lagos State Development Property Corporation	**1,200,000,000**								
282	077	Lagos Mortgage Board (LMB)	**4,000,000**					**1,819,300,780.84**		**35,112,993**	**70,990,000**
Lands			**26,503,000,000**	-	-	**648,960,126**	**462,624,526**	**1,000,000,000.00**	**1,000,000,000.00**	**5,938,526,921**	-
283	023	Lands Bureau	**23,000,000,000**			**389,379,761**	**315,000,000**	**1,000,000,000.00**	**1,000,000,000.00**	**1,539,635,044**	
284		Lands (Compensation)								**4,016,626,859**	
285	.018	Lagos State Valuation Office	**3,000,000**			**31,873,909**	**24,000,000**			**205,512,218**	
286	061	Office of the Surveyor-General	**3,500,000,000**			**227,706,456**	**123,624,526**			**176,752,800**	
Physical Planning and Urban Development			**28,710,742,227**	**500,000,000**	**500,000,000**	**1,418,350,902**	**100,000,000**	**6,110,000,000.00**	**6,110,000,000.00**	**13,384,737,676**	**943,440,858**
287	031	Ministry of Physical Planning and Urban Development	**500,000,000**			**1,418,350,902**	**100,000,000**	**110,000,000.00**	**110,000,000.00**	**2,319,644,869**	
288		Relocation of Mile 12 to Imota								**657,431,130**	
289		Relocation of Computer Village (Katangwa)								**164,518,263**	
290		MPPUD (Okobaba Resettlement)								**359,446,951**	
291	077	Lagos State Physical Planning Permit Authority(LASPPPA)	**25,133,142,227**							**1,486,526,311**	**195,520,858**
292	077	Lagos State Building Control Authority(LABCA)	**2,000,000,000**							**690,309,460**	**360,920,000**
293	077	Material Testing Laboratory Services	**900,000,000**							**335,089,746**	**200,000,000**
294	077	New Towns Development Authority		**500,000,000**	**500,000,000**			**6,000,000,000.00**	**6,000,000,000.00**	**459,860,027**	**100,000,000**
295	077	Lagos State Planning & Environmental Monitoring Authority (LASPEMA)	**9,600,000**							**39,965,130**	**37,000,000**
296	077	Lagos State Urban Renewal Authority (LASURA)	**168,000,000**							**5,679,250,286**	**50,000,000**
297		Slum Regeneration								**1,078,233,031**	
298		Redevelopment of Isalegangan								**114,462,474**	
Health			**1,221,120,000**	**10,961,375,000**	**10,961,375,000**	**37,338,524,275**	**18,152,731,427**	-	-	**33,270,367,812**	**6,245,694,273**
Health			**1,221,120,000**	**10,961,375,000**	**10,961,375,000**	**37,338,524,275**	**18,152,731,427**	-	-	**33,270,367,812**	**6,245,694,273**

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LAGOS STATE HOUSE OF ASSEMBLY
Y2020 APPROVED BUDGET
OMNIBUS

ANNEXURE II

			Y2020 REVENUE(CRF) N	Y2020 REVENUE DEDICATED (RECURRENT) N	Y2020 DEDICATED EXPENDITURE (RECURRENT) N	Y2020 PERSONNEL COST (MOET&P) N	Y2020 OVERHEAD COST N	Y2020 CAPITAL RECEIPTS (INFLOW) N	Y2020 CAPITAL DEVELOPMENT (OUTFLOW) N	Y2020 CAPITAL EXPENDITURE N	Y2020 SUBVENTION N
299	008	Ministry of Health	171,020,000			1,739,319,425	6,108,225,700			20,976,835,669	
300		Expansion/Acquisition of Properties for Hospitals								981,960,000	
301		Health Insurance Scheme/Equity Fund (MEPB)					7,433,041,344			-	
		Lagos State Blood Transfusion Service	44,700,000	11,175,000	11,175,000		52,450,996			137,710,000	
		Lagos State Health Monitoring and Accreditation Agency	140,000,000	60,000,000	60,000,000		80,000,000			143,265,790	
302		Lagos State Health Management Agency (LASHMA)								-	993,537,997
303		ICT for Health Insurance (LASHMA)								390,701,998	
304		Construction/Rehabilitation of Hospitals								2,544,365,808	
305		eHEALTH Platform for Health Insurance (LASHMA)								260,467,999	
306		Siemen Project								-	
307		Construction of Specialist Hospital								1,953,509,992	
308		Medical Emergency Preparedness Expenses					84,444,499			-	
309		Rural Posting Allowance for 6 General Hospitals								-	
310		MOH (Facility Management)					1,151,515,893			476,244,706	
311		Ministry of Health (LASUTH Project)								-	
312		LASUTH(Facility Mgt)								1,172,105,995	
313		Ministry of Health (LASUCOM Project)								817,431,821	
314	043	Health Service Commission	50,000,000	20,000,000	20,000,000	21,818,407,429	383,838,631			-	
315		HSC (Capacity Building)					1,535,354,524			-	
316	101	General Hospital, Lagos	35,000,000	665,000,000	665,000,000		67,446,577			-	
317	102	Gbagada General Hospital	65,000,000	585,000,000	585,000,000		82,652,671			-	
318	103	Orile Agege General Hospital	24,000,000	456,000,000	456,000,000		57,017,841			-	
319	104	Isolo General Hospital	25,000,000	475,000,000	475,000,000		45,825,670			-	
320	105	Ikorodu General Hospital	95,000,000	855,000,000	855,000,000		60,544,797			-	
321	106	Ajeromi General Hospital	30,000,000	270,000,000	270,000,000		48,381,263			-	
322	107	Badagry General Hospital	36,000,000	324,000,000	324,000,000		45,081,816			-	
323	108	Epe General Hospital	44,550,000	400,950,000	400,950,000		43,167,558			-	
324	109	Agbowa General Hospital	13,000,000	117,000,000	117,000,000		14,339,080			-	
325	111	Lagos Island Maternity Hospital	65,000,000	585,000,000	585,000,000		39,852,081			-	
326	112	Massey Street Children's Hospital, Lagos	4,250,000	80,750,000	80,750,000		74,779,471			-	

LAGOS STATE HOUSE OF ASSEMBLY
Y2020 APPROVED BUDGET
OMNIBUS

ANNEXURE II

			Y2020 REVENUE(CRF) N	Y2020 REVENUE DEDICATED (RECURRENT) N	Y2020 DEDICATED EXPENDITURE (RECURRENT) N	Y2020 PERSONNEL COST (MOET&P) N	Y2020 OVERHEAD COST N	Y2020 CAPITAL RECEIPTS (INFLOW) N	Y2020 CAPITAL DEVELOPMENT (OUTFLOW) N	Y2020 CAPITAL EXPENDITURE N	Y2020 SUBVENTION N
327	113	Mainland Hospital, Yaba	10,000,000	90,000,000	90,000,000		59,258,130			-	
328	114	Onikan Health Centre	20,000,000	180,000,000	180,000,000		12,073,119			-	
329	115	Apapa General Hospital	6,000,000	114,000,000	114,000,000		26,744,793			-	
330	116	Ebute-Metta Health Centre	22,000,000	198,000,000	198,000,000		36,912,361			-	
331	117	Harvey Road Health Centre	16,000,000	144,000,000	144,000,000		28,509,511			-	
332	118	Ketu-Ejinrin Health Centre	1,500,000	13,500,000	13,500,000		9,085,405			-	
333	119	Ijede General Hospital	25,000,000	225,000,000	225,000,000		16,215,404			-	
334	121	Ibeju-Lekki General Hospital	18,000,000	162,000,000	162,000,000		23,139,256			-	
335	122	Shomolu General Hospital	17,000,000	153,000,000	153,000,000		25,472,964			-	
336	124	Ifako/Ijaiye General Hospital	48,000,000	432,000,000	432,000,000		69,999,738			-	
337	125	Mushin General Hospital	30,000,000	270,000,000	270,000,000		41,761,035			-	
338	129	Surulere General Hospital	27,500,000	522,500,000	522,500,000		76,423,704			-	
339	131	Alimosho General Hospital	90,000,000	810,000,000	810,000,000		60,049,179			-	
340		Amuwo Odofin General Hospital	40,000,000	360,000,000	360,000,000		52,795,482			-	
341		Eti-Osa Maternal & Child care	7,500,000	142,500,000	142,500,000		52,795,482			-	
342		Lekki MCC								-	
343		Hospital Units (Dedicated)		100,000,000	100,000,000					-	
344		Hospital Units (IPP Intervention - Diesel)-(MEPB)					153,535,452			-	
345	077	Lagos State University College of Medicine(LASUCOM)		120,000,000	120,000,000					-	2,583,198,792
346		LASUCOM (Accreditation)								254,551,647	
347	077	Lagos State University Teaching Hospital (LASUTH)		2,000,000,000	2,000,000,000	6,906,330,356				716,286,997	1,291,599,396
348	077	Board of Traditional Medicine		20,000,000	20,000,000					82,484,640	12,419,225
349	077	Primary Health Care Board	100,000			6,874,467,065				1,953,509,992	620,961,248
350		Maintenance Support for PHC Centres								-	363,949,331
351		PHC (MSS/ Sure-P Nurses and CHEWS)								-	
352		PHC (Health Volunteer Workers)								-	
353	077	Lagos State AIDS Control Agency(LSACA)								356,841,158	280,674,484
354	077	Lagos State Accident & Emergency Centre								52,093,600	74,515,350
355		Lagos State Accident & Emergency Centre (Drugs and Medical Consumables)								-	24,838,450

ANNEX 4: OSUN STATE BUDGET EXTRACT

This annex contains a selection of health-related sections from the Osun state budget as extracted from State Government of Osun (2020). The recurrent and capital budgets are for the SMOH; similar pages exist for the SSHIS, Lautech Teaching Hospital, State Hospitals Management Board, and the PHC Board.

STATE GOVERNMENT OF OSUN

2020 ESTIMATES

OVERALL BUDGET SUMMARY

ADMINISTRATIVE CODES	MDAS	10000000 REVENUE	40000000 CAPITAL RECEIPTS (LOANS / BORROWINGS / EQUITY)	TOTAL INFLOW BUDGET	21000000 PERSONNEL	22000000 OTHER NON-DEBT RECURRENT	DEBT REPAYMENT
		₦	₦	₦	₦	₦	₦
	GOVERNANCE AND ADMINISTRATION SECTOR						
011100100100	OFFICE OF THE GOVERNOR	350,000,000.00	-	350,000,000.00	597,793,240.00	4,407,089,870.00	-
011100100900	OFFICE OF POLICY COORDINATION	-	-	-	-	-	-
011101000100	PUBLIC PROCUREMENT AGENCY	8,000,000.00	-	8,000,000.00	16,210,340.00	33,460,000.00	-
011101900100	MINISTRY OF SPECIAL DUTIES	1,000,000.00	-	1,000,000.00	3,943,680.00	10,878,820.00	-
011103800100	BUREAU OF PUBLIC SERVICE PENSION	10,000,000.00	-	10,000,000.00	9,031,543,220.00	2,643,960.00	-
011200300100	OSUN STATE HOUSE OF ASSEMBLY	2,200,000.00	-	2,200,000.00	423,785,650.00	1,280,803,940.00	-
011200400100	OSUN STATE HOUSE OF ASSEMBLY SERVICE COMMISSION	280,000.00	-	280,000.00	64,119,790.00	69,230,000.00	-
014000100100	OFFICE OF THE AUDITOR GENERAL (STATE)	2,000,000.00	-	2,000,000.00	113,965,290.00	12,172,990.00	-
014000200100	OFFICE OF THE AUDITOR GENERAL (LOCAL GOVERNMENTS)	435,700,000.00	-	435,700,000.00	68,733,860.00	12,690,990.00	-
012500500100	MINISTRY OF HUMAN RESOURCES & CAPACITY BUILDING	5,200,000.00	-	5,200,000.00	73,196,190.00	211,542,310.00	-
014700100100	CIVIL SERVICE COMMISSION	5,000,000.00	-	5,000,000.00	68,104,290.00	15,000,000.00	-
014700200100	LOCAL GOVERNMENTS SERVICE COMMISSION	407,936,500.00	-	407,936,500.00	44,384,810.00	400,000,000.00	-
014800100100	OSUN STATE INDEPENDENT ELECTORAL COMMISSION	10,000,000.00	-	10,000,000.00	70,584,850.00	1,500,000,000.00	-
022800100100	MINISTRY OF INNOVATION, SCIENCE AND TECHNOLOGY	20,000,000.00	-	20,000,000.00	72,778,650.00	32,385,000.00	-
048102100100	MINISTRY OF REGIONAL INTEGRATION	500,000.00	-	500,000.00	-	46,000,000.00	-
058100100100	MINISTRY OF LOCAL GOVERNMENTS AND CHIEFTAINCY AFFAIRS	50,000,000.00	-	50,000,000.00	59,512,360.00	12,990,000.00	-
	SUB-TOTAL GOVERNANCE AND ADMINISTRATION SECTOR	**1,307,786,500.00**	-	**1,307,786,500.00**	**10,705,655,920.00**	**8,056,224,880.00**	-
	AGRICULTURE SECTOR						
021800100100	MINISTRY OF AGRICULTURE, FOOD SECURITY	265,000,000.00	-	265,000,000.00	263,483,430.00	88,308,110.00	-
021801400100	OSUN PRODUCE BOARD	250,000,000.00	-	250,000,000.00	-	15,000,000.00	-
021810200100	OSUN STATE AGRICULTURAL DEVELOPMENT PROGRAMME	182,351,000.00	-	182,351,000.00	103,575,470.00	5,816,700.00	-
021810300100	OSUN STATE AGRICULTURAL DEVELOPMENT CORPORATION	41,000,000.00	-	41,000,000.00	83,761,260.00	10,093,760.00	-
	SUB-TOTAL AGRICULTURE SECTOR	**738,351,000.00**	-	**738,351,000.00**	**450,820,160.00**	**119,218,570.00**	-
	PLANNING AND BUDGET SECTOR						
023000300100	MINISTRY OF ECONOMIC PLANNING, BUDGET & DEVELOPMENT	1,240,150,000.00	-	1,240,150,000.00	82,303,770.00	263,472,760.00	-
023800400100	STATE BUREAU OF STATISTICS	200,000.00	-	200,000.00	33,148,010.00	35,380,000.00	-
	SUB-TOTAL PLANNING AND BUDGET SECTOR	**1,240,350,000.00**	-	**1,240,350,000.00**	**115,451,780.00**	**298,852,760.00**	-
	COMMERCE AND INDUSTRY SECTOR						
022200100100	MINISTRY OF INDUSTRY, COMMERCE & COOPERATIVES	200,000,000.00	-	200,000,000.00	142,775,130.00	109,803,620.00	-
022205100100	OSUN MICRO CREDIT AGENCY	20,250,000.00	-	20,250,000.00	-	8,060,040.00	-
023600100100	OFFICE OF TOURISM AND CULTURE	10,000,000.00	-	10,000,000.00	24,879,500.00	4,789,110.00	-
023600400100	OSUN STATE COUNCIL FOR ARTS AND CULTURE	10,000,000.00	-	10,000,000.00	39,728,750.00	10,500,000.00	-
023600200100	OSUN STATE TOURISM BOARD	109,469,000.00	-	109,469,000.00	11,506,750.00	5,083,800.00	-
061300100100	MINISTRY OF EMPOWERMENT AND YOUTH ENGAGEMENT	9,450,000.00	-	9,450,000.00	2,316,578,160.00	90,042,530.00	-
	SUB-TOTAL COMMERCE AND INDUSTRY SECTOR	**359,169,000.00**	-	**359,169,000.00**	**2,535,468,290.00**	**228,249,100.00**	-
	EDUCATION SECTOR						
051700100100	MINISTRY OF EDUCATION	2,030,000,000.00	-	2,030,000,000.00	401,371,900.00	421,860,790.00	-
051700300100	STATE UNIVERSAL BASIC EDUCATION BOARD	2,104,500,000.00	100,000,000.00	2,204,500,000.00	82,334,450.00	90,000,000.00	-
051700800100	OSUN STATE LIBRARY BOARD	1,000,000.00	-	1,000,000.00	8,102,020.00	708,880.00	-
051701000100	OSUN STATE MASS EDUCATION AGENCY	12,163,560.00	-	12,163,560.00	-	2,828,330.00	-
051701800100	OSUN STATE COLLEGE OF TECHNOLOGY, ESA-OKE	973,930,360.00	-	973,930,360.00	901,197,340.00	723,245,700.00	-
051701900100	OSUN STATE POLYTECHNIC, IREE	1,732,736,000.00	-	1,732,736,000.00	1,112,033,480.00	546,321,600.00	-
051702000100	OSUN STATE COLLEGE OF EDUCATION, ILESA	924,819,460.00	-	924,819,460.00	1,092,562,240.00	431,647,330.00	-
051702100100	OSUN STATE COLLEGE OF EDUCATION, ILA-ORANGUN	921,374,590.00	-	921,374,590.00	993,442,600.00	78,500,000.00	-
051702200100	OSUN STATE UNIVERSITY, OSOGBO	3,885,000,000.00	-	3,885,000,000.00	2,315,088,510.00	766,000,000.00	-
051702300100	LADOKE AKINTOLA UNIVERSITY OF TECHNOLOGY, OGBOMOSO	-	-	-	-	-	-
051702600100	OSUN CENTRAL EDUCATIONAL DISTRICT ILA ORANGUN (DISTRICT OFFICE)	80,000.00	-	80,000.00	1,312,633,660.00	88,000,000.00	-
051702700100	OSUN EAST EDUCATIONAL DISTRICT OFFICE, ILE - IFE (DISTRICT OFFICE)	211,640.00	-	211,640.00	1,190,385,680.00	88,000,000.00	-
051702800100	OSUN WEST EDUCATIONAL DISTRICT OFFICE, IKIRE (DISTRICT OFFICE)	375,000.00	-	375,000.00	1,101,474,610.00	87,500,000.00	-
051705100100	TEACHERS' ESTABLISHMENT AND PENSIONS OFFICE, OSOGBO	7,000,000.00	-	7,000,000.00	68,117,650.00	4,500,000.00	-
051705300100	BOARD FOR TECHNICAL AND VOCATIONAL EDUCATION	61,164,290.00	-	61,164,290.00	166,556,280.00	21,151,640.00	-
051706600100	OFFICE OF HIGHER EDUCATION, BURSARY & SCHOLARSHIP	10,000,000.00	-	10,000,000.00	19,606,970.00	10,000,000.00	-
051706500100	OSUN EDUCATION QUALITY ASSURANCE AND MORALITY AGENCY	20,000,000.00	-	20,000,000.00	-	5,000,000.00	-
	SUB-TOTAL EDUCATION SECTOR	**12,684,024,900.00**	**100,000,000.00**	**12,784,024,900.00**	**10,764,907,390.00**	**3,274,963,970.00**	-
	ENVIRONMENT SECTOR						
022205600100	OSUN SIGNAGE, HOARDING AND ADVERTISEMENT AGENCY	54,000,000.00	-	54,000,000.00	9,600,000.00	31,727,460.00	-
053500100100	MINISTRY OF ENVIRONMENT & SANITATION	444,000,000.00	-	444,000,000.00	60,265,140.00	24,900,000.00	-
053500200100	OSUN PARKS AND GARDENS MANAGEMENT AGENCY	10,000,000.00	-	10,000,000.00	-	20,000,000.00	-
053505300100	OSUN STATE WASTE MANAGEMENT AGENCY	150,000,000.00	-	150,000,000.00	98,209,930.00	70,846,180.00	-
023305100100	OFFICE OF FORESTRY, NATURAL & MINERAL RESOURCES	500,000,000.00	-	500,000,000.00	89,123,790.00	8,460,650.00	-
	SUB-TOTAL ENVIRONMENT SECTOR	**1,158,000,000.00**	-	**1,158,000,000.00**	**257,198,860.00**	**155,934,290.00**	-
	HEALTH SECTOR						
052100100100	MINISTRY OF HEALTH	200,055,000.00	-	200,055,000.00	388,315,970.00	200,000,000.00	-
052100200100	OSUN STATE HEALTH INSURANCE SCHEME	2,975,954,500.00	-	2,975,954,500.00	56,106,110.00	22,917,810.00	-
052102800100	LAUTECH TEACHING HOSPITAL, OSOGBO	500,000,000.00	-	500,000,000.00	4,855,834,090.00	225,000,000.00	-
052110200100	OSUN STATE HOSPITALS MANAGEMENT BOARD	60,000,000.00	-	60,000,000.00	2,245,857,400.00	110,000,000.00	-
052111600100	PRIMARY HEALTH CARE DEVELOPMENT BOARD	2,103,600,000.00	-	2,103,600,000.00	71,809,140.00	171,452,170.00	-
	SUB-TOTAL HEALTH SECTOR	**5,839,609,500.00**	-	**5,839,609,500.00**	**7,417,622,710.00**	**729,369,980.00**	-
	INFRASTRUCTURE SECTOR						
022905300100	OFFICE OF THE TRANSPORTATION	1,000,000,000.00	-	1,000,000,000.00	167,930,710.00	34,007,900.00	-
023400100100	MINISTRY OF WORKS & TRANSPORT	3,000,000,000.00	-	3,000,000,000.00	161,424,600.00	187,223,360.00	-
023400200100	OFFICE OF THE SURVEYOR - GENERAL	300,000,000.00	-	300,000,000.00	21,811,990.00	6,230,500.00	-
023400400100	OSUN ROAD MAINTENANCE AGENCY	3,703,700.00	-	3,703,700.00	55,201,840.00	8,000,000.00	-
023405500100	OSUN ASSETS MANAGEMENT AGENCY	200,000,000.00	-	200,000,000.00	-	12,690,990.00	-
025305300100	OSUN STATE PROPERTY DEVELOPMENT CORPORATION	150,000,000.00	-	150,000,000.00	48,907,910.00	31,088,520.00	-
025305600100	OSUN STATE CAPITAL TERRITORY DEVELOPMENT AUTHORITY	30,000,000.00	-	30,000,000.00	19,220,800.00	12,690,990.00	-
025305800100	OSUN NEW TOWNS AND GROWTH AREAS DEVELOPMENT AUTHORITY	321,800,000.00	-	321,800,000.00	-	15,863,730.00	-
026000100100	MINISTRY OF LANDS AND PHYSICAL PLANNING	1,800,000,000.00	-	1,800,000,000.00	29,481,760.00	15,863,730.00	-
	SUB-TOTAL INFRASTRUCTURE SECTOR	**6,805,203,700.00**	-	**6,805,203,700.00**	**553,979,610.00**	**293,659,720.00**	-
	SOCIAL DEVELOPMENT AND WELFARE SECTOR						
012400100100	MINISTRY OF HOME AFFAIRS	80,600,000.00	-	80,600,000.00	128,945,930.00	190,000,000.00	-
061400100100	MINISTRY OF WOMEN & CHILDREN AFFAIRS	2,900,000.00	-	2,900,000.00	72,701,680.00	182,310,000.00	-
053900100100	MINISTRY OF SOCIAL PROTECTION, SPORTS & SPECIAL NEEDS	1,000,000.00	-	1,000,000.00	38,389,510.00	30,000,000.00	-
053905100100	OSUN STATE SPORTS COUNCIL	10,000,000.00	-	10,000,000.00	133,624,140.00	70,000,000.00	-
	SUB-TOTAL SOCIAL DEVELOPMENT AND WELFARE SECTOR	**94,500,000.00**	-	**94,500,000.00**	**370,622,230.00**	**442,310,000.00**	-
	SECURITY LAW AND JUSTICE SECTOR						
031801100100	JUDICIAL SERVICE COMMISSION	3,000,000.00	-	3,000,000.00	63,849,100.00	63,454,920.00	-
032600100100	MINISTRY OF JUSTICE	79,965,000.00	-	79,965,000.00	267,131,790.00	105,738,200.00	-
032605100100	THE JUDICIARY (HIGH COURT OF JUSTICE)	120,000,000.00	-	120,000,000.00	485,996,060.00	317,274,630.00	-
032605200100	CUSTOMARY COURT OF APPEAL	35,000,000.00	-	35,000,000.00	403,125,380.00	157,805,530.00	-
	SUB-TOTAL SECURITY LAW AND JUSTICE SECTOR	**237,965,000.00**	-	**237,965,000.00**	**1,220,102,330.00**	**644,293,280.00**	-
	INFORMATION AND COMMINICATION SECTOR						
012300100100	MINISTRY OF INFORMATION AND STRATEGY	5,000,000.00	-	5,000,000.00	61,760,280.00	6,345,490.00	-
012300300100	OSUN STATE BROADCASTING CORPORATION	307,300,000.00	-	307,300,000.00	218,808,790.00	112,019,790.00	-
	SUB-TOTAL INFORMATION AND COMMINICATION SECTOR	**312,300,000.00**	-	**312,300,000.00**	**280,559,040.00**	**118,365,280.00**	-
	WATER AND SANITATION SECTOR						
028200100100	OFFICE OF WATER RESOURCES, RURAL AND COMMUNITY AFFAIRS	6,940,000.00	1,854,209,460.00	1,861,149,460.00	66,861,660.00	5,000,000.00	-
028210200100	OSUN STATE WATER CORPORATION	6,450,000,000.00	3,000,000,000.00	9,450,000,000.00	487,147,080.00	60,000,000.00	-
028210300100	RURAL WATER & ENVIRONMENTAL SANITATION AGENCY	226,600,000.00	460,738,400.00	687,335,400.00	67,981,850.00	2,000,000.00	-
	SUB-TOTAL WATER AND SANITATION SECTOR	**6,683,540,000.00**	**5,314,944,860.00**	**11,998,484,860.00**	**621,990,560.00**	**67,000,000.00**	-
	FINANCE & REVENUE MOBILIZATION SECTOR						
022000100100	MINISTRY OF FINANCE	1,951,900,000.00	-	1,951,900,000.00	998,928,080.00	8,315,708,280.00	-
022000200100	DEBT MANAGEMENT OFFICE	-	-	-	5,062,960.00	-	1,200,000,000.00
022000700100	OFFICE OF THE ACCOUNTANT - GENERAL	55,125,200,000.00	-	55,125,200,000.00	128,162,760.00	70,000,000.00	-
022000800100	OSUN STATE INTERNAL REVENUE SERVICE	11,651,594,830.00	-	11,651,594,830.00	38,658,580.00	190,364,770.00	-
	SUB-TOTAL FINANCE & REVENUE MOBILIZATION SECTOR	**68,728,694,830.00**	-	**68,728,694,830.00**	**1,170,812,350.00**	**5,576,073,050.00**	**1,200,000,000.00**
	TOTAL EXPENDITURE	**105,889,494,430.00**	**5,414,944,860.00**	**111,304,439,290.00**	**36,465,191,230.00**	**20,004,514,850.00**	**1,200,000,000.00**

2020 SUMMARY OF PERSONNEL BUDGET

052100100100 - MINISTRY OF HEALTH

ECONOMIC CODE	DESCRIPTION	APPROVED BUDGET 2020	APPROVED BUDGET 2019
		₦	₦
2	**EXPENDITURES**		
21	**PERSONNEL COST**		
2101	**SALARY**		
210101	**SALARIES AND WAGES**		
21010101	SALARY	364,853,290.00	577,360,400.00
	SUB-TOTAL SALARIES AND WAGES	**364,853,290.00**	**577,360,400.00**
2102	**ALLOWANCES AND SOCIAL CONTRIBUTION**		
210201	**ALLOWANCES**		
21020101	NON REGULAR ALLOWANCES	23,462,680.00	25,643,630.00
	SUB-TOTAL ALLOWANCES	**23,462,680.00**	**25,643,630.00**
	TOTAL TRANSFERS	**388,315,970.00**	**603,004,030.00**

DETAILS OF OTHER RECURRENT BUDGET (OVERHEAD COST AND CFRC)

052100100100 - MINISTRY OF HEALTH

ECONOMIC CODE	DESCRIPTION	APPROVED BUDGET 2020 ₦	APPROVED BUDGET 2019 ₦
22	**OTHER RECURRENT COSTS**		
2202	**OVERHEAD COST**		
220201	**TRAVEL& TRANSPORT - GENERAL**		
22020101	LOCAL TRAVEL & TRANSPORT: TRAINING	-	3,000,000.00
22020102	LOCAL TRAVEL & TRANSPORT: OTHERS	3,000,000.00	
	SUB-TOTAL TRAVEL& TRANSPORT - GENERAL	**3,000,000.00**	**3,000,000.00**
220202	**UTILITIES - GENERAL**		
22020202	TELEPHONE CHARGES	1,000,000.00	
	SUB-TOTAL UTILITIES - GENERAL	**1,000,000.00**	**-**
220203	**MATERIALS & SUPPLIES - GENERAL**		
22020301	OFFICE STATIONERIES / COMPUTER CONSUMABLES	2,000,000.00	2,000,000.00
22020307	DRUGS/LABORATORY/MEDICAL SUPPLIES	27,000,000.00	66,100,000.00
	SUB-TOTAL MATERIALS & SUPPLIES - GENERAL	**29,000,000.00**	**68,100,000.00**
220204	**MAINTENANCE SERVICES - GENERAL**		
22020401	MAINTENANCE OF MOTOR VEHICLE / TRANSPORT EQUIPMENT	38,900,000.00	44,100,000.00
22020405	MAINTENANCE OF PLANTS/GENERATORS	1,000,000.00	
22020411	MAINTENANCE OF COMMUNICATION EQUIPMENTS	-	752,650.00
	SUB-TOTAL MAINTENANCE SERVICES - GENERAL	**39,900,000.00**	**44,852,650.00**
220205	**TRAINING - GENERAL**		
22020501	LOCAL TRAINING	5,000,000.00	
	SUB-TOTAL TRAINING - GENERAL	**5,000,000.00**	**-**
220207	**CONSULTING & PROFESSIONAL SERVICES - GENERAL**		
22020708	MEDICAL CONSULTING	5,000,000.00	681,400.00
	SUB-TOTAL CONSULTING & PROFESSIONAL SERVICES - GENERAL	**5,000,000.00**	**681,400.00**
220208	**FUEL & LUBRICANTS - GENERAL**		
22020801	MOTOR VEHICLE FUEL COST	39,100,000.00	500,000.00
22020802	OTHER TRANSPORT EQUIPMENT FUEL COST	-	500,000.00
22020803	PLANT / GENERATOR FUEL COST	-	500,000.00
	SUB-TOTAL FUEL & LUBRICANTS - GENERAL	**39,100,000.00**	**1,500,000.00**
220210	**MISCELLANEOUS EXPENSES GENERAL**		
22021002	HONORARIUM & SITTING ALLOWANCE	16,000,000.00	13,000,000.00
22021003	PUBLICITY & ADVERTISEMENTS	-	9,160,000.00
22021004	MEDICAL EXPENSES-LOCAL	40,000,000.00	55,100,000.00
22021008	SUBSCRIPTION TO PROFESSIONAL BODIES	-	11,000,000.00
22021014	ANNUAL BUDGET EXPENSES AND ADMINISTRATION	-	200,000.00
22021042	RECURRENT ADJUSTMENT	-	3,250,000.00
	SUB-TOTAL MISCELLANEOUS EXPENSES GENERAL	**56,000,000.00**	**91,710,000.00**
2305	**OTHER CAPITAL EXPENDITURE**		
230501	**OTHER CAPITAL EXPENDITURE**		
23050102	COMPUTER SOFTWARE ACQUISITION	-	20,350,000.00
23050103	MONITORING & EVALUATION	5,000,000.00	
23050104	ANNIVERSARIES/ CELEBRATIONS	17,000,000.00	15,738,240.00
	SUB-TOTAL OTHER CAPITAL EXPENDITURE	22,000,000.00	36,088,240.00
	TOTAL OTHER RECURRENT COSTS	**200,000,000.00**	**245,932,290.00**

DETAILS OF CAPITAL BUDGET

052100100100 - MINISTRY OF HEALTH

ECONOMIC CODE	DESCRIPTION	APPROVED BUDGET 2019	APPROVED BUDGET 2020
		₦	₦
3	**ASSET**		
31	**CURRENT ASSET**		
310501	**INVENTORIES**		
31050102	MEDICAL STORES	180,000,000.00	30,060,000.00
31050135	OTHER STOCK	20,153,600.00	-
	SUB-TOTAL INVENTORIES	**200,153,600.00**	**30,060,000.00**
32	**NON-CURRENT ASSETS**		
320101	**LAND & BUILDING - GENERAL**		
32010101	LAND & BUILDINGS - ADMINISTRATIVE	200,000,000.00	139,973,980.00
	SUB-TOTAL LAND & BUILDING - GENERAL	**200,000,000.00**	**139,973,980.00**
320104	**FIXED ASSETS - GENERAL**		
32010405	MOTOR VEHICLES	25,000,000.00	20,000,000.00
	SUB-TOTAL FIXED ASSETS - GENERAL	**25,000,000.00**	**20,000,000.00**
320105	**OFFICE EQUIPMENT - GENERAL**		
32010501	COMPUTERS/NETWORKING EQUIPMENT	6,000,000.00	4,000,000.00
32010502	PRINTERS	1,000,000.00	1,000,000.00
32010503	SCANNERS	1,000,000.00	1,000,000.00
32010504	FAX MACHINE	2,000,000.00	-
32010505	PHOTOCOPIERS		2,000,000.00
32010507	SHREDDING MACHINES	500,000.00	-
32010508	PROJECTORS	2,000,000.00	-
	SUB-TOTAL OFFICE EQUIPMENT - GENERAL	**12,500,000.00**	**8,000,000.00**
320106	**FURNITURE & FITTINGS - GENERAL**		
32010601	CHAIRS	1,000,000.00	1,000,000.00
32010602	TABLES	1,000,000.00	1,000,000.00
32010604	TELEVISION SETS	375,000.00	-
32010606	AIR -CONDITIONER	1,000,000.00	-
	SUB-TOTAL FURNITURE & FITTINGS - GENERAL	**3,375,000.00**	**2,000,000.00**
	TOTAL ASSET	**441,028,600.00**	**200,033,980.00**